

How to Use the “Administrator Orientation”

This workbook is organized to include 13 orientation topics with sessions & activities to complete.

1. Read the “Introduction” section first.
2. Review the “Key Contact List” and verify all items are obtained or accessible.
3. Review the “Orientation Checklist: Administrator” for activities and sessions to complete. This is the **primary document to record** your orientation. As you complete each session, initial the appropriate sections. Upon completion, maintain in the teammate file.
4. Complete Orientation Session #1 & Orientation Session #2.
5. Complete additional sessions in any order.
6. Complete the activities & worksheets provided each week.
7. Note any questions or points of clarification to review with the designated specialist from the LaVie Care Team at the end of each session.

Administrator Orientation

Introduction	Introduction to the Role of Administrator Orientation Instructions
Orientation Session #1	Getting to Know Your Team, Facility & Key Systems/Processes
Orientation Session #2	The Clinical Product / Nursing, Social Services, Nutrition Services, Quality of Life
Orientation Session #3	Finance: Budgets, Reports & Accounts Payable
Orientation Session #4	Reimbursement, Ancillary Services & Medical Review
Orientation Session #5	Business Office Services
Orientation Session #6	Census Development & Admissions Process
Orientation Session #7	Human Resources & Staff Development
Orientation Session #8	Compliance
Orientation Session #9	Customer Satisfaction
Orientation Session #10	Legal Issues & Risk Management
Orientation Session #11	Information Technology
Orientation Session #12	Environmental / Plant Operations
Orientation Session #13	Regulatory Compliance
Completion	Complete & Sign the "Orientation Checklist: Administrator"

Introduction

Whether you are a first time Administrator, or have many years of experience, it's true that the first few weeks of a new job can be overwhelming. You'll be receiving a lot of new information on business operations, processes and getting to know a lot of new people.

This orientation manual is designed to help you become familiar with some of the systems, processes and tools available to you in your role as the new Administrator of your facility. You'll also begin identifying key resources & organizational tools that you'll need to do your job.

Sessions are provided to help you navigate the orientation process. As you work through each session you'll find important information and activities designed to help you become familiar with the facility and the resources available to you.

As you work through each session, ask yourself **"What are the opportunities to improve the quality of work and improve the outcomes being achieved?"** Finding ways to maintain a quality product, or improve on an existing system are critical to running a successful skilled nursing & rehab facility.

Sessions do not need to be completed in order; however, you will find it helpful to complete **Orientation Session #1 & #2** first as information in these sessions will be referred to in other sessions.

Review & complete the learning activities. As you complete, record any questions you may have. Review your questions with your teammate specialists.

At the end of each session, discuss what you learned with the specialists and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Before you start orientation, it may be helpful to review some basic information about your facility. For some Administrators, this may be the first time you've worked in a facility that is designated as a "Limited Liability Company" or "LLC".

Introduction

Following are some important points to know about working for a Limited Liability Company (LLC):

- An LLC is a standard business structure, but is relatively new to the long-term care environment.
- An LLC is defined as a “single purpose entity”. The facility’s “LLC” name is the entity’s legal name. In this example, the legal name is “ABC Healthcare Associates, LLC” (doing business as (d.b.a.) ABC Health & Rehab).
- All agreements must be in the name of the Legal Entity
- Your facility is a Key Entity. It is a separate and stand alone LLC
- The LLC is the licensee and operates the facility.
- Facility teammates, including the Administrator, work for the facility LLC and are not teammates of the management company.
- Your facility has an agreement with a management company, which is similar to an executive recruiter. Under this agreement, the management company appoints the Administrator to the facility.
- The management company does not own or operate the facility and does not “manage” the facility. This company provides management services only.
- Your facility contracts with:
 - Management Company for management services
 - LaVie Administrative Services for “back office” services (e.g., payroll, A/P, IT etc)
 - LaVie Rehabilitation, LLC for rehab services

Management specialists will be available to assist you with becoming familiar with facility policies, procedures, systems and processes.

Introduction

Getting Started with Orientation

- ✓ Review the “Key Contact List” and verify all items are obtained or accessible.
- ✓ Review the “Orientation Checklist: Administrator”. This is the form to document completion of all of the orientation sessions and will go into your teammate file upon completion.
- ✓ Complete **Orientation Session #1: Getting to Know Your Team, Facility & Key Systems & Processes**
- ✓ Complete **Orientation Session #2: The Clinical Product / Nursing, Therapy Services, Social Services, Nutrition Services & Quality of Life**
- ✓ Plan a schedule to complete the additional Orientation Sessions
- ✓ Attend “**General Orientation**” & Complete the “**Orientation Education/In-Service Record for Teammates**”
- ✓ Complete the “**Internal Control Letter**” immediately upon hire (Refer to Session #5 for a copy of the letter)
- ✓ Notify **survey office** with the name of the new Administrator.

Key Contacts

LAVIE RESOURCES	CONTACT INFORMATION
Accounts Payable	Jami Hurst, VP of Accounts Payable 770.730.1132 jhurst@laviecare.com Michael Langan, AP Manager 770.730.1140 mlangan@laviecare.com
Benefits/Payroll	Mark Linn, Director of Payroll & Benefits 813.744.2828 mlinn@laviecare.com
Internet/Portal Access	LaVie Administrative Services IT Help Desk – 800.830.4213 Enter help ticket on web: http://helpdesk.laviecare.com
Name tag & Business cards	Alan Cosby, VP of Communications 770.730.1111 acosby@laviecare.com
Team (HR)	Janelle Fairbrother, SVP of Team & Benefits 978.809.0603; jfairbrother@laviecare.com Kayla Stanley, Team & Benefits Coordinator 770.730.1180 kstanley@laviecare.com Ken Hawkins, VP Florida Division 321.750.7888 khawkins@laviecare.com Dave Kendrick, VP National Division & Rehab 813-495-5382 dkendrick@lavierehab.com
Travel	Angela Forcino, Director of Travel & Event Mgmt 813.744.2868 aforcino@laviecare.com

VENDOR RESOURCES	CONTACT INFORMATION
<u>Alpha Graphics:</u> P&P Manuals & Laminates/Tools (previously Franklin's; Ben Print)	Ronnie Townsend & Barbara Heslin 813.630.1655 rtownsend@alphagraphics.com ; bheslin@alphagraphics.com
<u>Briggs:</u> Forms	Barb Keylor 800.247.2343 ext.8757 Keylor.barb@briggscorp.com
Current LTC Survey Manual	American Health Care Association (AHCA) www.ahca.org

Introduction

Orientation Checklist Instructions

1. As you review and complete items listed for a session, sign and date **each** line.

SAMPLE OF CORRECT SUBMISSION

Learning Activities	ADM Signature when Complete	Date
Introduction		
Completed General Orientation	<i>John Doe</i>	4/25/2011
Read LLC information	<i>John Doe</i>	4/25/2011
Review & Complete Internal Control Letter (refer to Session #5)	<i>John Doe</i>	4/25/2011
Notify survey office of name of new Administrator (refer to Session #13)	<i>John Doe</i>	4/25/2011
Session #1: Getting to Know Your Team, Facility & Key Systems/Processes		
Completed the "Worksheet: Get to Know Your Team"	<i>John Doe</i>	4/25/2011
Scheduled time to meet staff on all shifts	<i>John Doe</i>	4/25/2011
Reviewed current staffing schedule & process for staffing	<i>John Doe</i>	4/25/2011
Completed the "Worksheet: Get to Know Your Facility"	<i>John Doe</i>	4/25/2011

SAMPLE OF INCORRECT SUBMISSION

Learning Activities	ADM Signature when Complete	Date
Introduction		
Completed General Orientation	<i>John Doe</i>	4/25/2011
Read LLC information		
Review & Complete Internal Control Letter (refer to Session #5)		
Notify survey office of name of new Administrator (refer to Session #13)		
Session #1: Getting to Know Your Team, Facility & Key Systems/Processes		
Completed the "Worksheet: Get to Know Your Team"	<i>John Doe</i>	4/25/2011
Scheduled time to meet staff on all shifts		
Reviewed current staffing schedule & process for staffing		
Completed the "Worksheet: Get to Know Your Facility"		
Completed the "Meeting Calendar"		
Completed the "Worksheet: Specialist Resources"		
Completed the "Worksheet: Portal Resources & Applications"		
Can identify which reports are received daily & weekly & demonstrate how to use information		

2. Once all pages of the checklist have been signed and dated, remove from packet, scan and upload as training to Ad Hoc on LaVie University.
www.lavieuniversity.com
 - a. Name the file using the following example: "YYYY Onboarding LAST name FIRST initial Example: 2011Onboarding Fey D"

Orientation Checklist: Administrator

Learning Activities	ADM Signature when Complete	Date
Introduction		
Completed General Orientation		
Read LLC information		
Review & Complete Internal Control Letter (refer to Session #5)		
Notify survey office of name of new Administrator (refer to Session #13)		
Session #1: Getting to Know Your Team, Facility & Key Systems/Processes		
Completed the "Worksheet: Get to Know Your Team"		
Scheduled time to meet staff on all shifts		
Reviewed current staffing schedule & process for staffing		
Completed the "Worksheet: Get to Know Your Facility"		
Completed the "Meeting Calendar"		
Completed the "Worksheet: Specialist Resources"		
Completed the "Worksheet: Portal Resources & Applications"		
Can identify which reports are received daily & weekly & demonstrates how to use information		
Can identify where to find policies and procedures, forms, & vendor contacts		
Sent introductory letters to Residents, Families, Resident Council & Family Council		
Attend the RM/QI Meeting		
Verify all P&P Manuals in use are up-to-date		
Verify State Operational Manual is up-to-date		
Session #2: The Clinical Product		
Completed the "Worksheet: Current Clinical Picture"		
Completed the "Worksheet: Operations & Clinical Care Management"		
Met with the Director of Nursing & Reviewed the Clinical Programs Manual, Clinical Indicators		
Met with the Social Services Director & Reviewed the Social Services Manual		
Met with the Nutrition Service Manager & Dietitian & Reviewed the Nutrition Services Manual		
Met with the Activity Coordinator & Reviewed the Recreation & Therapeutic Activities Manual		
Met with the Director of Rehab & Reviewed clinical Rehab manual		

Orientation Checklist: Administrator

Learning Activities	ADM Signature when Complete	Date
Session #3: Finance: Budgets, Reports & Accounts Payable		
Demonstrates how to access financial reports on the Portal		
Demonstrates how to enter "key labor" data daily		
Reviewed financial reports & labor reports		
Demonstrates how to access the budget on the Portal		
Reviewed budget		
Demonstrates where to locate the Critical Vendor List		
Met staff accountants		
Describes how to submit a Capital Expenditure Request (CER)		
Demonstrates how to use the A/P Select system		
Demonstrates how to use the Expense Tracking system		
Session #4: Reimbursement, Ancillary Services & Medical Review		
Completed the " Worksheet: Reimbursement Challenge "		
Reviewed existing vendor contracts with therapy, pharmacy, food vendors, hospice, physician advisors, medical director, specialty beds, medical supplies		
Read and reviewed Hospice policies & procedures in Administrative Manual		
Session #5: Business Office Services		
Completed "Internal Control Letter"		
Completed the " Worksheet: Business Office Services "		
Attended Credit & Collections Training		
Session #6: Census Development & Admission Process		
Completed the " Worksheet: Census Development & Admission "		
Observed the Admission Process from referral through admission		
Reviewed the Discharge Management process with the interdisciplinary team		
Reviewed the facility Sales & Marketing Plan		

Orientation Checklist: Administrator

Learning Activities	ADM Signature when Complete	Date
Session #7: Human Resources & Staff Development		
Completed the “Worksheet: Human Resources”		
Completed the “Leadership in LTC” course on the On-line university		
Has reviewed the HR P&P Manual		
Has a copy of the HR Forms Manual		
Has reviewed the Education Resource Manual		
Has reviewed the Employee Handbook		
Has reviewed the Safety & OSHA Compliance Manual		
Has reviewed current union contracts, if applicable		
Demonstrates where to find HR related information on the Portal		
Reviewed the education calendar with the Regional Clinical Nurse Educator		
Reviewed the Development Feedback Tool		
Session #8: Compliance		
Demonstrates use of the Compliance / HIPAA Portal site		
Demonstrates use of the OIG Website		
Completed Compliance Training P7003 course with the On-line university		
Completed the Compliance Training Post Test		
Completed and signed the Code of Ethics & Business Conduct Program		
Demonstrates understanding of role of Facility Compliance Officer		
Session #9: Customer Satisfaction		
Completed the “Worksheet: Customer Satisfaction”		
Reviewed the “Commitment to Excellence” Customer Service Program		
Reviewed the “Guardian Angel” program in the Administrative Manual		
Reviewed the “Grievance process” in the Social Services Manual		
Reviewed the “Resident Council” process in the Activities Manual		
Met with the President of the Resident Council & discussed issues/concerns		
Reviewed the “Family Council” process in the Social Services Manual		
Reviewed the results of the most recent Customer Satisfaction Surveys from residents, families and staff		
Reviewed the LaVie Mission & Core Values		

Orientation Checklist: Administrator

Learning Activities	ADM Signature when Complete	Date
Session #10: Legal Issues & Risk Management		
Read the “Contract Review” process in the Administrative Manual & Portal		
Read the “Abuse Prevention Program” in the Administrative Manual		
Read the “Incident/Accident” process in the Administrative Manual		
Reviewed the “Incident Listing & Logging” application on the Portal		
Review the “Risk Management/Quality Improvement” process in the Administrative Manual		
Attend the Risk Management/ Quality Improvement meeting in the facility & review focus areas for improvement		
Session #11: Information Technology		
Contacted LaVie IT Helpdesk & completed training		
Session #12: Environmental / Plant Operations		
Completed the “Worksheet: Life Safety Review ”		
Conducted facility tour with Maintenance Director & management specialist		
Reviewed the current TELs Exception Report		
Completed TELs Training through Direct Supply		
Confirmed that the facility Disaster Plan & Emergency Call List is current		
Completed the F323 Accident Prevention training program on the portal		
Reviewed the facility Safety Manual		
Verified a complete list of permits & visual inspections has been completed		
Has identified and reviewed all system generated reports		
Developed a prioritized plan to address identified environmental, plant op and equipment issues		
Session #13: Regulatory Compliance		
Completed the “Worksheet: The Long Term Care Survey”		
Reviewed survey results for the past 3 years		
Reviewed the “5 Star ” status of facility & identified opportunities & plan for improvement		
Has contacted the local survey office & met surveyors		
Letter sent to survey office notifying of new Administrator		
Describes survey activity & communication notification requirements		

NOTES

Orientation Session #1

Getting to Know Your Team, Facility & Key Systems/Processes

Introduction

For this first session, you'll spend time getting to know your staff and becoming familiar with some of the language and culture of the facility. You'll also begin identifying key resources, organizational tools and systems/processes that you'll need to do your job.

Learning activities have been provided to assist you in learning the key skills needed for your role as the Administrator. Review & complete the learning activities. As you complete, record any questions you may have.

At the end of each session, review any questions you may have with the specialists from your management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do to Complete this Session

- ✓ Complete the **"Worksheet: Get to Know Your Team"**
 - Schedule time to meet with staff on all shifts
 - Review the current staffing schedule and the process for staffing
- ✓ Complete the **"Worksheet: Get to Know Your Facility"**
 - Complete the "Meeting Calendar" that cues you into the key meetings that keep the operation running smoothly
- ✓ Complete the **"Meeting Calendar"**
- ✓ Complete the **"Worksheet: Portal Resources & Applications"**
- ✓ Complete the **"Worksheet: Specialist Resources"**

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from your management company
- ✓ Complete **"Session #1"** of the **Orientation Checklist**

Orientation Session #1

Getting to Know Your Team, Facility & Key Systems/Processes

Getting to Know Your Team

The successful operation and management of a skilled nursing facility relies on a group of people who are knowledgeable about their jobs, able to communicate well with each other and able to effectively resolve conflicts and problems. They share a common vision and goal in providing care and services to residents, patients and families.

What to Do

- ✓ Complete the **“Worksheet: Get to Know Your Team”** with each department
- ✓ Schedule time to meet staff on all shifts
- ✓ Review the current staffing schedule and process for staffing

Getting to Know Your Facility

Knowing your facility is important in order to provide perspective and context to your role. Understanding the facility regulatory history and current status, facility layout and operational goals can help you focus your initiatives and goals.

What to Do

- ✓ Complete the **“Worksheet: Get to Know Your Facility”**
- ✓ Complete the **“Meeting Calendar”**
- ✓ Discuss the Customer Service Expectations of the facility with your RVP and specialists
- ✓ Tour the facility, making note of areas that may require additional follow-up

Orientation Session #1

Getting to Know Your Team, Facility & Key Systems/Processes

Getting to Know Your Resources

You'll find you have quite a few resources to refer to in your role as an Administrator in this facility. The management company provides specialists in many areas of business operations. Get to know those teammates. Also, learn how to use the portal. This is an important tool for accessing important information and resources. The portal is a source for data management, reports, tools and educational material.

What to Do

- ✓ Complete the **"Worksheet: Specialist Resources"**
- ✓ Complete the **"Worksheet: Portal Resources & Applications"**
- ✓ Identify where to find policies and procedures, reports, applications, department specific information, forms and more
- ✓ Read the **"Risk Management/ Quality Improvement"** policy in the Administrative Manual
- ✓ Attend a **"Risk Management/Quality Improvement"** meeting

Orientation Session #1

Getting to Know Your Team, Facility & Key Systems/Processes

Reports

Reports are generated based on data submitted by your facility to LaVie Administrative Services. Most reports are generated either daily or weekly to assist in monitoring and managing your business.

Following are examples of some of the most common reports. Times may be subject to change.

Report	Frequency
Cash report / facility MTD	Daily / 9:00 a.m.
Cash report / exception	Daily / 9:00 a.m.
Census	Daily / noon
Key Factor Labor Report	Daily / Noon
Summary Statement of Operations	Monthly
Detail Statement of Operations	Monthly
Detail Trail Balance	Monthly
Payroll Report – Extended Version	Every 2 weeks
Expense Tracking System	Daily
GMA System	Daily
Medicaid / current pending days over 45	Q Monday / 9:00 a.m.
Medicaid / current pending unpaid days	Q Monday / 9:00 a.m.
Medicaid / all active summary report	Q Thursday / 8:00 a.m.
Referral Log / weekly exception report	Q Friday / 11:00 a.m.
Clinical Indicators	Q Friday / 1:00 a.m.
Medicaid / weekly exception report	Q Friday / 4:00 p.m.

Additional Reports / Discuss with your Management Company Specialist

- Adverse Incident Reporting
- LaVie Care Alert Line
- TELs Activity

Worksheet: Get to Know Your Team

Instructions: Complete this worksheet during the first week of orientation. Fill in the name of the team member, the date you met with him/her. Recommendations for discussion are provided. Upon completion of your meeting, ask the team member to initial.

The Director of Nursing is: _____ Since: _____ Information Reviewed: • Current clinical outcomes • Survey history • Staffing model • Incident/Accident process • Clinical challenges • Clinical Indicators		_____ Initials _____ Date
Notes: 		

The Business Office Manager is: _____ Since: _____ Information Reviewed: • Process for checking claims before submission • Process for Medicaid applications • AP Processing • Credit & Collection processes • Weekly meetings / expectations • Checks & balances		_____ Initials _____ Date
Notes: 		

The Payroll Coordinator is: _____ Since: _____ Information Reviewed: • Payroll and benefits processes • Payroll distribution dates • New hire process		_____ Initials _____ Date
Notes: 		

The MDS Coordinator is: _____ Since: _____ Names of additional MDS Staff: _____		_____ Initials _____ Date
Information Reviewed: • How the MDS schedule and work is managed • Where MDS calendar is kept & how it's updated • Who is the backup MDS coordinator?		
Notes: 		

Worksheet: Get to Know Your Team

The Assistant Director of Nursing/Unit Managers are: _____ Since _____ Information Reviewed: - Types of residents/patients cared for on the unit - Open positions and strategy in use for filling - Process for new hires (orientation, TB test, drug test, Hep B offer) & how it's documented - Where employee educational files are kept and how they are updated - Process the UM uses to monitor care being delivered - Process used for communicating with MD & family - Review education calendar and status of completion - Process for validating clinical skills on hire and annually	_____ Initials _____ Date
Notes:	

The Social Service Director is: _____ Since: _____ Information Reviewed: - Process for seeing new admissions and scheduling follow up visits - Process for managing discharges - Process for managing grievances and current outstanding grievances - Process for updating advance directives	_____ Initials _____ Date
Notes:	

The Activity Director is: _____ Since: _____ Information Reviewed: - Process for evaluating new admissions and re-evaluating existing residents for appropriate activities - How programming is adapted to individual preferences - How many volunteers? - Review Activity Calendar	_____ Initials _____ Date
Notes:	

Worksheet: Get to Know Your Team

The Nutrition Services Manager is: _____ Since: _____ The Dietitian is: _____ Since: _____ Information Reviewed: • Meal times • How kitchen is notified of resident food preferences • How special dietary needs are met • Process for obtaining meals after kitchen closes • The primary dining style in the dining rooms • All the locations that food is served in the facility (dining room, restorative, resident rooms) & % of residents eating in each location • Process for managing snack/hydration carts Notes:	_____ Initials _____ Date
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The Admission Director and/or Coordinator is: _____ Since: _____ Information Reviewed: • Information provided to new admissions • How new admissions are processed • Process for room assignments Notes:	_____ Initials _____ Date
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The Therapy Director is: _____ Since: _____ Information Reviewed: • Names & titles of therapy staff • Process for evaluating new admissions • Process for scheduling therapy appointments • How therapy participates in daily meetings & care plans • # of Days Therapy service is available • Hours available • How to contact therapy staff after hours Notes:	_____ Initials _____ Date
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The Admission Specialist is: _____ Since: _____ Information Reviewed: • Marketing plan and referral targets • Perception from physicians/hospitals/community of the facility staff ability to care for residents/patients being admitted • Process for communicating potential referral information to the DON and nursing staff Notes:	_____ Initials _____ Date
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Worksheet: Get to Know Your Team

The Environmental Services (maintenance, laundry, housekeeping) is: _____ Since: _____ Information Reviewed: • Hours and availability • Schedule for cleaning • How routine maintenance is tracked and monitored • What types of equipment is cleaned and managed in house	_____ Initials _____ Date
Notes:	

Additional Staff & Team Members

Name & Title	Notes

Worksheet: Get to Know Your Facility

Instructions: Complete this worksheet during the first week of orientation. Make a note of any questions to review with your Specialist.

Emergency Preparedness Plan
• Locate and review the Emergency Preparedness Plan for the facility • Record the name of the facility(s) where residents would be evacuated to: _____ • Record the name of the transportation company contracted to provide evacuation services: _____ • Date of Last Review: _____ • Review the Fire Safety & Evacuation plan. Review the location of smoke compartments, exits and fire extinguishers
Notes:

Regulatory Status																																
What is the current regulatory status of the facility? ☐ Substantial Compliance ☐ Not currently in compliance Describe: _____																																
Regulatory History: Record the results of the last 3 Standard Surveys																																
<table border="1"><thead><tr><th>Date of Survey</th><th>Total # Citations</th><th># & Type of Substandard Citations</th><th>Remedies</th></tr></thead><tbody><tr><td> </td><td> </td><td> </td><td> </td></tr><tr><td> </td><td> </td><td> </td><td> </td></tr><tr><td> </td><td> </td><td> </td><td> </td></tr></tbody></table>	Date of Survey	Total # Citations	# & Type of Substandard Citations	Remedies																												
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Regulatory History: Record the results of Complaint Surveys with Citations in the past 3 years																																
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Notes:																																

Worksheet: Get to Know Your Facility

Census Mix

Where is census mix information kept? _____

What is the total number of licensed beds in the facility? _____

What is the Average Length of Stay (ALOS): _____

What is the average age of the residents/patients in this facility? _____

Current Census	#Medicare	#Medicaid	#Managed Care	#Private Pay

Notes:

Vendors

Vendor	Name	Phone Number	Notes
Pharmacy			
Support Surfaces			
Medical Supplies			
Oxygen Concentrators & Portable Oxygen			
Hospice			
Laboratory Services			
X-Ray Services			
Transportation			

LAVIE MEETING STANDARDS

	Meeting	Time/Duration	Attendees	Order	Required Materials/ Record Keeping
DAILY MEETINGS	1. Clinical Review	8AM 30 minutes	- DON - ADON/Unit Mgrs. - Social Services - Nutrition Services - MDSC - Activities	Unit by unit review at unit stations	Review of: 24 Hr Report - MAR/TAR - Labs - Physician order changes - High risk medications - Chart Review, as needed
	2. Daily Operations (Stand-Up)	9AM 30 minutes	- ADM - Department Heads	- Key operational data/ Census, Payer Mix - Admissions/Discharges - Visitors/Tours - Staffing - Grievance Guardian Angel rounds - Outstanding cites/POC - Quick review of departmental events - Risk Management changes	Daily Ops Agenda for follow-up
	3. Daily Utilization Review	9:30AM Dependent on Medicare & Managed Care Census: - Less than 20: 15 minutes 21+ : 30 minutes to 1 hour	- ADM - MDSC - DON - DOR - BOM	- Review all r/p receiving therapy - Refusals, withheld treatments, MD appointments - Authorization; payer verification - ARD reviewed - All payors receiving therapy reviewed - Any further clinical information from 8AM Clinical Review meeting	- MDSC records - DOR records - Chart Review, as needed - Casamba notes
	4. Care Review (Care Plan)	- Within 72 hrs. for short-stay patient - Within 5 days for long-term resident - Scheduled ongoing as needed through stay; at least quarterly 72 hrs. prior to discharge	- ADM - DON - Social Services - Resident/Patient - Family - MDSC (for quarterly care plan meeting)	- Review of resident/patient goals, care plan, family expectations - Coordinate care planning process to occur during this meeting	- Chart - Care Review - Care Management Summary
WEEKLY MEETINGS	1. Weekly Utilization Review	Dependent on MCA/ Managed Care Census: - Less than 10: 15 min 11-20: 30 minutes 21+: 1 hour	- ADM - MDSC - DON - DOR - Unit Mgrs.	- Review all r/p in therapy to include status/condition and impact on care delivery - Authorization; payer verification - ARD; planned discharge; day of their stay they are on; when 100th day is - ICD9 code alignment - R/P progress in therapy, or lack thereof - Review clinical aspects of r/p care that may impact reimbursement and overall r/p status/condition - Documentation review that supports skilled service	- Chart - ADL flow sheet - Weekly Utilization Log - Match Meeting Checklist (obtain from BOM to review & return)
	2. Care Management - Weekly - Monthly - Quarterly	Small: 3 to 4 hours Large: 4 to 6 hours	- ADM - DON - Unit Mgrs. - Social Services - Dietary Mgr - Activities - DOR (case load only) - RD (case load only)	- IDT reviews the resident/patient to determine progress towards goals and update interventions as appropriate. - Schedule residents/patients receiving therapy first; schedule resident/patients at Nutrition Risk second.	- Chart - Care Management Summary
	3. Medicaid Pending	30 minutes	- ADM - DON - BOM - Social Services - Admissions for pre-admins	Review of status and manage Medicaid Pending process	- Medicaid Pending Log - Medical tracking system on portal
	4. ADM/BOM Meeting	30 minutes to 1 hour	- ADM - BOM	Refer to Effective Meetings Policy	- Interim Aging - Other various reports - ADM/BOM Meeting Report
	5. Weekend Meeting		- ADM - Weekend Mgr.	Meet every Friday to discuss expectations for the weekend	
MONTHLY MEETINGS	1. Match Meeting	Dependent on Medicare & Managed Care Census: - Less than 20: 30 min 21+: 1 hour	- ADM - MDSC - DOR - BOM	Verification of accuracy of all billing materials	- UB04 Validation Reports - Ancillaries - ICD 9 - Hospital dates - ARDs - RUG levels - Managed Care levels - Medicare Match Meeting Form
	2. RM/QI	60 minutes	- ADM - Medical Director - Department Heads	- Review of key outcome data and reports for quality improvement process - Customer satisfaction & opportunity for improvement - Incidents/Accidents requiring further resolution - Tag files; Action plans; Survey outcomes - Compliance	- Incident/Accident Log - Grievance Log - Tag File - RM/QI Minutes (portal link) - Compliance Agenda/minutes - Infection Control logs - Pharmacy issues
	3. Safety Meeting	30 minutes to 1 hour	Permanent Members: Maintenance & HR - Rotating members include reps from several departments, multiple job types from all shifts. One member is appointed the Chairperson	- Communicate hazards, accident trends, safety violations - Recommend corrective action to the RM/QI committee	- Safety Committee Binder - Safety Inspection Checklist - Safety Committee Minutes Form
	4. Town Hall/Team Meetings		All teams/All shifts	- News - Teammate recognition - Listening session	Agenda

LaVie Standards / Facility Administrator

Daily
8:00am Facility Walk Through & Prep for Day
9:00—9:30am Daily Operations "Stand Up" Meeting / Follow Daily Operations Agenda
9:30—10:15am Daily Utilization Meeting
10:15—11:30 Review Labor Schedule to Budget; Review Expense Tracking System to Budget; Follow up on outstanding issues & incident resolution; Return calls; Meet with Department Leaders as needed; Conference Calls
11:30—1:00 On the Floor; Dining Room; Therapy Gym
1:00—2:30 Lunch & Office time; Conference Calls
2:30—5:00pm On the Floor
Meet with Family members as available

Weekly	Wk 1	Wk 2	Wk 3	Wk 4
Contact / Connect with high risk family members				
Introduce & Kickoff the Care Management Meeting				
Check in with Regional Development Specialist on Sales activity & plan for the week. Communicate with the RDS daily if census is not to budget.				
Meet with Medical Director & Director of Nursing; Review Clinical Indicators; Conduct rounds with DON on Thursday to determine residents at risk for RTH over weekend				
Meet with new teammates / spend time on the floor with teammates hired in last 90 days; Verify clinical training is occurring with new teammates by nursing team				
Review training & orientation schedule with the RCNE				
Meet with DOR to assure efficiencies and teamwork in resident transport are working; Review LaVie Grand Rounds.				
Co-Lead Weekly Utilization Review with MDSC				
Review Medicaid Pending Process				
Meet with Business Office Manager to review collections process				
Call with Regional HR/Team to discuss progress towards filling open positions				
Meet with Social Services Director & Activities Director to review outstanding issues, provide guidance on plans & activities				
Walk through kitchen; observe meal time & tray line; Meet with Nutrition Services Manager to provide guidance on food cost management and quality improvement activities				
Eat a meal at the facility				
Conduct a detailed review of facility environment & physical plant with Maintenance Director. Review facility needs & projects				
Meet with Manager on Duty Friday to discuss expectations for the weekend				
Schedule evening hours, to include meal time, at a minimum 1 x per week, more as needed				

Monthly	Initial when complete
Conduct Grand Rounds with Department Leaders	
Review Sales & Marketing Plan with the RDS	
Review AR balances with Business Office Manager	
Review P&L	
Attend Match Meetings to verify accuracy of all billing materials	
Lead Risk Management / Quality Improvement Meeting	
Review education calendar and scheduled training/education for the month	
Review forced ranking and spend 1:1 time with high performing teammates	
Participate / Lead Safety Committee Meeting	
Attend Resident Council at minimum once a quarter; Attend Family Council	
Schedule weekend hours per Manager on Duty schedule	
Town Hall/Teammate meeting / All shifts / Teammate recognition	

Facility Name: _____ **ADM Name:** _____ **Date:** _____

Calendar

Instructions: Review the **LaVie Meeting Standards & LaVie FA Standards** to determine how these meetings are scheduled.
Record the frequency, time and where they are held.

• Daily Operations Meeting • Care Review • Daily Utilization Meeting • Clinical Review	• Care Management Meetings (Weekly, Monthly & Qtly) • Weekly Utilization Review • Medicaid Pending • ADM/BOM Meeting • Weekend Meeting	• Match Meeting • Risk Management/ Quality Improvement • Safety Meeting • Town Hall/Team Meetings
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Sunday	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday

Worksheet: Portal Resources & Applications

References & Resources	Where You'll Find It	Initial to Indicate Review
"Portal" <i>This site is hosted by LaVie Administrative Services and contains online applications & resources provided by the contracted management company for your facility</i>	1. Log-in to the computer & click on "Portal" icon 2. Enter your username & password 3. Click on the "bars" along the side to explore the site. 4. Remember how to access this site, you'll be directed to come back during orientation	Initials Date
Policies & Procedures <i>Available online and to order as hard copy.</i> <i>The Document Catalog houses policies and procedures for business office services, administration, clinical services and human resources.</i>	1. Access the "Portal" (see above) 2. Hover over the bar labeled "P&P"; you'll notice 3 selections: Accounts Payable, Document Catalog & Payroll. Click on "Accounts Payable" & "Payroll" • Review the list of available policies and forms under AP. Click to open and review. To Access Document Catalog online: • Click on "Document Catalog" • P&P can be viewed by either selecting the "Tree View" or the "Search View" • If select "Tree View", click on the company division and then click on whichever department manuals you'd like to view (i.e., Business Office, Clinical Services, etc.) • Click on "Policy" • Select the manual you would like to view and click on it • Select the policy/procedure you would like to review and click on it • Opens as a PDF file, able to print • If select "Search View", enter key words into the search bar. If more than one word in a search, use double quotation marks or separate the words with AND or OR To Order Manuals: • Click on "Order Manuals Here" • Fill in Alpha Graphics online order form • Orders are shipped to your facility	I have reviewed the document catalog and have an updated version of the manuals available to me. Initials Date
Reports (RPTS) <i>Reports provide key operational information</i>	1. Access the "Portal" (see above) 2. Click on the side bar labeled "Reports" 3. Review available reports and select desired report. Available reports include: GL; PR; AP; AR; Cash; Spec. 4. See sidebar on the right side of the portal for additional reports available. These include: Medicaid rates; LTC month end reports; AR reports; Labor; Average length of stay; Clinical benchmarks These reports need to be reviewed in detail with Regional Vice President (RVP) and/or VP Finance/Controller.	I have reviewed the "Reports" and understand how to access available reports Initials Date

Worksheet: Portal Resources & Applications

Applications (APPS) <i>Applications are where data is entered and recorded for reports.</i>	1. Access the "Portal" (see above) 2. Click on the side bar labeled "APPs" 3. Review each application by clicking on it and reading the description of the application. Applications include: AP Purchasing; AR billing; Budgets; Calendar; Clinical; Dietary; Marketing; Ops; PR; Risk management; Scorecard 4. Review each of the applications and follow directions for accessing the tool. 5. Contact your management specialist with any questions.	I have reviewed the "Reports" and understand how to access available reports Initials Date
Departments (DEPTS) <i>A variety of information is available on these sites including tools, forms and educational material.</i>	1. Access the "Portal" (see above) 2. Click on the side bar labeled "DEPTS" 3. Click on each department listed and review the information and content available. Departments include Business Office, Clinical Support, Communication, Compliance, Customer Svc Training, HIPAA & regulatory updates; HR; IT; Legal; Nutrition Support; Risk & Insurance; Ops and Travel & Event management	I have reviewed the "Departments" and understand how to find department specific information Initials Date
Documentation Forms	Forms referenced in the clinical policies and procedures are accessible as follows: • Forms with an "FSE" code or other numerical code are ordered from Briggs. Notify Central Supply for ordering forms • Forms labeled as "copy form" are original forms for printing & copying at the facility, not available to order from Briggs All forms are available for viewing on the "Portal". • Access the "Portal" and "Document Catalog" as described above • Select "Search View": enter a key word describing the type of form	I have reviewed the portal and the forms available. Initials Date

Worksheet: Specialist Resources

The following teammates are available to your facility from your management company. Once completed, keep this worksheet with you as a quick reference to know where to locate a resource.

Specialist	Name	Phone Number	Notes
Regional Vice President (RVP)			
Regional Business Specialist (RBS)			
Regional Clinical Nurse (RCN)			
Regional Clinical Nurse Educator (RCNE)			
Regional Reimbursement Specialist (RRS)			
Team Specialist (TS)			
Regional Director of Admissions (RDA)			

Additional Vendor Specialists

Therapy Vice President			
Consultant Pharmacist			
Pharmacy Manager			
Dietitian			

NOTES

NOTES

Orientation Session # 2

Understanding the Clinical Product

Introduction

For this orientation session, you'll focus on learning about the current status of the clinical product in your facility and how you can use your tools and resources to improve clinical outcomes.

What is the Clinical Product?

As the Administrator, you are responsible for the overall execution of systems & services in your facility. The clinical product includes all clinical services provided to residents and patients in your facility. Clinical Services includes:

- Nursing
- Therapy
- Social Services
- Nutrition Services
- Activities

For this session, you'll learn the process for collecting information on resident/patient outcomes, compare it against expected clinical standards and analyze whether or not the clinicians in your facility are working together to achieve positive patient outcomes. You'll learn how to identify areas that require improvement and begin forming a plan to improve the clinical product you are providing.

Learning activities have been provided to assist you in learning the key skills needed for your role as the Administrator. Review & complete the learning activities. As you complete, record any questions you may have.

At the end of this session, review any questions you may have with your specialist and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session # 2

Understanding the Clinical Product

What to Do To Complete this Session

- ✓ Complete the “**Worksheet: Current Clinical Picture**”
- ✓ Complete the “**Worksheet: Operations & Clinical Care Management**”
- ✓ Meet with the **Director of Nursing**. Review and discuss the **Clinical Programs Manual**.
- ✓ Meet with the **Social Services Director**. Review and discuss the **Social Services Manual**.
- ✓ Meet with the **Nutrition Services Manager** and the **Dietitian**. Review and discuss the **Nutrition Services Manual**.
- ✓ Meet with the **Activities Director**. Review and discuss the **Recreational & Therapeutic Activities Manual**.
- ✓ Meet with the **Director of Rehab Services** to review the clinical care process in the rehab department.

When Sessions are Completed

- ✓ Review completed sessions with the Regional Clinical Nurse from the management company
- ✓ Complete “**Session #2**” of the **Orientation Checklist**

Orientation Session # 2

Understanding the Clinical Product

Current Clinical Picture

In this section, you'll learn how to identify the clinical data available to you and to analyze the results of this data.

What to Do

- ✓ Complete the "**Current Clinical Picture**" worksheet

In order to complete this worksheet, you'll need to be able to access the current clinical data available. Your teammates will be able to show you how to access the available data.

Data you'll need to obtain for this section includes:

- ✓ **Clinical Indicators**
Clinical information is collected and entered into this portal application on a weekly basis by a designated person from the facility. Usually this is the Director of Nursing or designee. If you are not sure how this information is currently being collected and reported, ask your Quality Specialist. In order to complete the "Current Clinical Picture" worksheet, you'll need to obtain the report from this week and the past 2 weeks, for a total of 3 weeks of data.
- ✓ **Quality Indicators/Quality Measures**
The QI/QM information for your facility is posted on the Nursing Home Compare website that is maintained by the Centers for Medicare & Medicaid Services (CMS). This information is collected by submitted MDS and CASPER information. It's important to make yourself familiar with the information on this website. Residents and families also have access to this information and may ask you questions based on this information.

Please note: QI/QM will not be available until 2012 due to the transition to MDS 3.0

Orientation Session # 2

Understanding the Clinical Product

Clinical Programs & Clinical Documentation

You learned about some of the resources available to you last session. This session you'll spend time getting to know more of the specifics about the some of the clinical resources available to you. Schedule some time to meet with your Director of Nursing to review the activities and worksheets below.

What to Do

- ✓ Locate the "Bookshelf" on the portal.
- ✓ Access the "Document Catalog" on the Portal, or review a copy of the policies and procedures in your facility
- ✓ Review the "**Clinical Programs Manual**" with your Director of Nursing

Clinical Programs are the core policies, procedures and documentation forms available to your nursing staff to assist in conducting assessments, developing a plan of care and providing guidance for the frequency of re-evaluating your resident/patient's progress towards goals.

The Clinical Programs have a similar design and structure which meet best practice clinical standards as well as state and federal regulatory requirements.

Each program includes:

- Overview
- Guidelines for conducting assessments and observations
- Guidelines for selecting and implementing interventions to address identified issues
- Guidelines for developing and evaluating a care plan
- References as applicable

Orientation Session # 2

Understanding the Clinical Product

Admission

Clinical admission documentation should be completed within 8 hours after the resident/patient is admitted. Unless the resident/patient is admitted right at shift change, there are initial documents that should be completed by the nurse admitting the resident/patient.

Documents to complete immediately include:

- Admission/Re-Admission Data Collection and Initial Care Plan
- MAR/TAR
- Code Status

By completing this initial documentation, you'll likely be able to determine the Clinical Programs that will be required to provide comprehensive nursing care to the resident/patient. The Admission/Re-Admission Data Collection and Initial Care Plan forms provide guidance to the nursing staff in identifying the additional clinical documentation that will be required, based on how they answer the questions on the form.

Every resident/patient admitted to the facility is assumed to be at risk for falls. Successful fall prevention is identifying the reasons that may put someone at greater risk for falls and implementing interventions to address those specific reasons.

Assessing the condition of the resident/patient skin is very important and must be done as soon as possible after the resident/patient arrives at the facility. Identifying existing skin impairments and looking for the clinical indicators that may indicate if a skin breakdown is imminent, are important to document and implement a treatment plan to address these issues as soon as possible.

Additional Admission Assessments

In addition to assessments completed by the nursing team, additional admission evaluations and assessments are completed by therapy, nutrition services, social services and therapeutic recreation. This is an important time to determine the initial level of care the resident/patient will require and assist the interdisciplinary team in getting to know the resident/patient and family.

Orientation Session # 2

Understanding the Clinical Product

Clinical Programs

Following is a brief review of the clinical programs available and recommendations for when to implement these programs for residents/patients.

Activity: Review the following clinical programs with the Director of Nursing and initial on the table when complete.

Clinical programs are available on the portal or by ordering from Alpha Graphics (See Session #1)

Clinical Programs	Implement for Residents/Patients when:	I've reviewed this program (Initial when complete)
Admission/Re-Admission	Initiated on admission and/or re-admission	
Clinical Care Management	Guidelines for the clinical management of care	
Behavior Intervention & Management	Demonstrating behaviors as a result of a psychiatric diagnosis, mental illness or cognitive impairment	
Bowel & Bladder Program	Incontinence of bladder or bowel is identified. This program contains information on urinary catheter management as well.	
Depression Management	Demonstrating symptoms of depression	
Elopement Prevention & Management	- Has a cognitive impairment, but able to move independently - Exit seeking behavior - History of elopement	
Fall Risk Prevention & Management	This program is implemented for all residents/patients. It includes prevention as well as guidelines for evaluating a resident/patient post fall	
Medication Management	Medication regime is managed & monitored to promote and/or maintain r/p highest practicable, mental, physical & psychosocial well being	
Nutrition/Hydration Management	This program is implemented for all residents/patients.	
Pain Management	Presence of pain or a diagnosis that indicates a likelihood of pain	
Restorative Nursing - Activities of Daily Living - Transfers & Bed Mobility - Ambulation - Contracture Prevention - Dining	Preparing for discharge from therapy and additional nursing support is needed to assist r/p in attaining or maintaining highest practicable level	
Restraint Prevention & Management	Considering a restraint, for any reason This program also provides a side rail assessment and guidance for developing safe use of side rails	
Skin Prevention & Wound Care Management	This program is implemented for all residents/patients. It includes prevention as well as guidelines for treatment of wounds	
Change in Condition	Assess r/p clinical status when change identified	
Documentation	Guidelines for documenting	
Discharge Management	D/C planning begins at time of admission & continues throughout r/p stay	

Orientation Session # 2

Understanding the Clinical Product

Activity

Meet with the Director of Therapy Services, Social Services Director, Nutrition Services Director and Dietitian and the Activity Director. Plan enough time to review the policy and procedure manual together.

Department	Recommended Review	Initial when complete
Therapy Services Director of Rehab Services	- Policies & Procedures - Process for evaluating and treating residents/patients - Documentation forms used - Outcome reports provided	
Social Services Social Services Director	- Social Services Manual - Process for assessing newly admitted residents and conducting ongoing assessments - Documentation forms used - Discharge plan	
Nutrition Services - Nutrition Services Manager - Dietitian	- Nutrition Services Manual - Process for assessing newly admitted residents and conducting ongoing assessments - Documentation forms used	
Activities Activity Director	- Recreational & Therapeutic Activities Manual - Process for assessing newly admitted residents and conducting ongoing assessments - Documentation forms used	

Orientation Session # 2

Understanding the Clinical Product

Operations & Clinical Care Management

Operating a skilled nursing facility is a complex undertaking. It's a business that requires consistent customer care and service to be delivered by skilled, competent and compassionate staff. It's a business where the receipt for reimbursement is complicated and labor intensive and it's a business that is held to heavy and complicated regulatory standards through a strict enforcement process.

The successful operation of a skilled nursing facility requires:

- ✓ Smart & competent leaders
- ✓ Consistent focus & application of fundamental business practices
- ✓ Consistent application of strategies to reduce risk
- ✓ Quick & correct response to variances

These standards are provided to the Administrator and Director of Nursing for directing **operations and clinical management practices** that are crucial to operating a successful skilled nursing facility.

The standards provided here are a compilation of best practices observed and tested by Administrators and Directors of Nursing working in the industry.

Before You Start this Section

In order to complete the activities provided, you'll need access to the following resources:

- ✓ Administrative Manual
- ✓ Business Office Manual
- ✓ Clinical Reimbursement Manual
- ✓ Credit & Collections Manual
- ✓ Clinical Programs Manual

Orientation Session # 2

Understanding the Clinical Product

Core Operations Management Meetings

There are a number of regular and routine meetings that, as the Administrator, you may use to manage your business. Some of the core operations management meetings are described here.

You'll find additional reference information about each of these meetings in the following manuals.

Operations Management Meetings	Additional Source for Information
Daily Operations	<i>Administrative Manual</i>
Daily Utilization Review	<i>Administrative & Clinical Reimbursement Manual</i>
Weekly Utilization Review	<i>Administrative & Clinical Reimbursement Manual</i>
Match Meeting	<i>Business Office Manual</i>
Medicaid Pending	<i>Credit & Collections Manual</i>
Risk Management/ Quality Improvement	<i>Administrative Manual</i>

Orientation Session # 2

Understanding the Clinical Product

Daily Operations

Go to the Administrative Manual and review the following:

- ✓ Daily Operations

The Daily Operations meeting is a brief meeting, held every morning no later than 9am. This meeting should last approximately 20-30 minutes. If the meeting is consistently lasting longer than this, review the process with the Regional Vice President (RVP) to identify opportunities for improved efficiencies. As the Administrator, you are responsible for providing your staff with education on what the purpose of this meeting is and how to come prepared to deliver the key operational data that is needed.

Daily operations meeting may be combined with Daily Clinical Review on weekends. Weekend meeting attendees include, but are not limited to Weekend Manager, Nursing Supervisor and Therapist.

Who Should Attend Daily Operations?

Attendees at the Daily Operations Meetings should include, but are not limited to the Administrator, all department leaders or designee.

Daily Operations Meeting Agenda

The agenda guides your Daily Operations meeting, making sure to cover the key operational information and data.

Review the “Daily Operations Meeting Agenda” on the following page and answer the following questions.

- ✓ Which team member should be expected to provide updates on census?
- ✓ Which team member can provide information on reimbursement?
- ✓ Who provides updates on Admissions/Discharges / Room Changes & Transportation?
- ✓ Who provides updates on Staffing & Teammate Issues?
- ✓ Who provides information on risk management issues?

Discussion Point	Notes
Census	In-house: _____ Bedholds: _____
Payer Mix	Medicare A: _____ Part B: _____ Mgd. Care: _____ Medicaid: _____ Hospice: _____ VA: _____ Private Pay: _____ Payer changes: _____
Admissions	Confirmed: _____ Pending: _____
Discharges:	
Room Changes	
Transportation Needs	
MDS Assessments	Quantity & Type of uncompleted assessments: Entries _____ Discharge _____ OBRA _____ Medicare _____ EOT/SOT _____ OT hours required to get them completed ☐ No ☐ Yes, Amount _____
Staffing:	Budgeted Labor/OT hours & % Open Positions: 90-Day Development Feedback due: New teammate hire:
Incidents / Accidents	Verify MD & family notification Resolved on e-portal
Grievances	
Guardian Angel Rounds	
Outstanding Regulatory Citations	
Notes	

Orientation Session # 2

Understanding the Clinical Product

Daily Utilization Review

Go to the Clinical Reimbursement Manual and review the following:

- ✓ Daily Ancillary Expense Management
- ✓ Daily PPS Log

This is a brief, daily meeting immediately after the Daily Operations Meeting and should last approximately 15-20 minutes. This meeting is done to review current residents/patients to assure appropriate utilization of care and services. Changes in the types of care and services being provided are communicated.

This is an important meeting in managing resources to assure that residents/patients are receiving the right care and services for the right amount of time.

Who Should Attend Daily Utilization Review?

Attendees at the Daily Utilization Review should include, but are not limited to the Administrator, MDS Coordinator, Director of Nursing, Director of Rehab, Social Services & Business Office Manager

Daily Ancillary Management Guide

This is a tool that can be helpful in cueing discussion regarding status of care and services being provided to residents/patients.

Review the “Daily Ancillary Management Guide” on the following page and answer the following questions

- ✓ What is important information to review for a patient with Medicare Part A?
- ✓ If a resident is requiring the use of a low-air loss bed, what questions are important to ask?
- ✓ When a patient is receiving IV antibiotics, what are the important parameters to understand how to manage this treatment?

Orientation Session # 2

Understanding the Clinical Product

Daily Ancillary Management Guide

Rehabilitation Services	IV Medications Fluids/Parenteral Feeding	Complex Medical Rental/Lease	Diagnostics	Outpatient Services Consolidated Billing
- Part A - Optimal Assessment Reference Date - Capturing Extensive Services - Part B - Identified reason for treatment - Supportive Documentation - Frequency - Duration - Managed Care - Authorized level of service - Monitor utilization of minutes - Update Auth# per contract - Medicaid/Other - Reason for treatment - Frequency - Duration - Monitor utilization of minutes	- Classification - Stop/Start dates - Possible alternate medications or routes - Communication to Physician on progress - Follow up labs per Physician order and protocol	- Type - Low Air Loss/ Alt Pressure Beds - Communication to Rehab dept on progress if applicable - Communication to Physician on progress - Follow Physician protocol related to purpose and duration/usage - Rented or Leased	- Specific Type - Purpose and results of diagnostics - Frequency reoccurring or single order - Communication to Physician on necessity to continue	- If possible identify service to be provided pre-admission - Excluded Service SNF not responsible if provided in hospital or hospital based unit - Cardiac Cath - CT Scans - MRI - Ambulatory with operating room - Radiation - Angiography - Certain Lymphatic/Venous procedures - Some Chemo items & services - Some Radioisotope services - Some customized prosthetic devices - Facility responsibility for included services - Communication to provider prior to service - Letter of Agreement in place - Agreed pricing based on current fee schedule - Frequency/Duration of treatment

Orientation Session # 2

Understanding the Clinical Product

Weekly Utilization Review

Go to the Administrative & Clinical Reimbursement Manual and review the following:

- ✓ Weekly Utilization Review Meeting

The Weekly Utilization Review is a weekly meeting and is a more in-depth review of current residents/patients to assure that the care and services required of the resident/patient are being met, that the medical record documentation reflects the care and services provided and that correct and proper submission for reimbursement occurs. **This meeting may be combined with the Weekly Care Management meeting.**

This is an important meeting in managing resources to assure that residents/patients are receiving the right care and services and that the facility is billing correctly for the care and services provided.

Who Should Attend Weekly Utilization Review?

Attendees at the Weekly Utilization Review should include, but are not limited to the Administrator, MDS Coordinator, Director of Nursing, Director of Rehab, Unit Managers

Match Meeting

Go to the Medicare Manual and review the following:

- ✓ Match Meeting

The Match Meeting is conducted monthly to verify the accuracy of documents that are required to meet regulatory standards and professional practice standards. This is a quality assurance process to verify documents are correct and match. Documents reviewed in the Match Meeting include, but may not be limited to:

- | | |
|--------|------------------|
| ✓ UB04 | ✓ Ancillaries |
| ✓ RUG | ✓ ICD-9 codes |
| ✓ ARD | ✓ Hospital dates |

Who Should Attend the Match Meeting?

Attendees at the Match Meeting should include, but are not limited to the Administrator, MDS Coordinator, Director of Rehab & Business Office Manager

Orientation Session # 2

Understanding the Clinical Product

Medicaid Pending

Go to the Credit & Collections Manual and review the following:

- ✓ Medicaid Pending Process

The Medicaid Pending meeting is conducted weekly to determine progress of Medicaid Pending applications. Team verifies that every resident/patient has updated comments added to their file in the Medical Tracking System located on the portal. The Medicaid Pending Log is reviewed to verify steps in the process are being followed.

Who Should Attend Medicaid Pending Meetings?

Attendees at the Medicaid Pending Meeting should include, but are not limited to the Administrator, Business Office Manager & Social Services. The Admission Director and/or Coordinator is involved pre-admission until initial process is completed.

Risk Management/ Quality Improvement (RM/QI)

Go to the Administrative Manual and review the following:

- ✓ Risk Management / Quality Improvement Program

This meeting is conducted monthly and directed by the Administrator. Key outcome data and reports are reviewed for quality improvement progress or opportunities for continued improvement. Customer service feedback should be reviewed & responded to and incidents/accidents reviewed and verified for appropriate response. Tag files, Action Plans and Survey responses are reviewed for progress towards improvement.

The RM/QI Meeting is most effective when the team can focus on the results of process improvement teams and offer recommendations for further improvement. Work done by action teams, or process improvement teams should be summarized and completed prior to the RM/QI meeting.

Who Should Attend the RM/QI Meeting?

State Regulatory requirements may dictate how many people and which staff members are required to participate in the quality improvement meeting.

Attendees at the RM/QI Meeting should include, but are not limited to the Administrator, Director of Nursing, MDS Coordinator, Director of Rehab, Social Services, Nutrition Services Manager, Activity Director , Admissions, Business Office Manager, Marketing, Maintenance.

Review the “Monthly RM/QI Committee Meeting” tools on the following pages for meeting agenda items and minutes form. Documents are also posted on the portal.

MONTHLY RM/QI COMMITTEE MEETING

Meeting Date: _____

AGENDA

(Topics for Discussion)

- Activities
 - Admissions
 - Business Office
 - Clinical
 - Customer Service
 - Education
 - Health Information Services
 - Environmental/Plant Operations
 - MDS
 - Medical Director
 - Nutrition Services
 - Regulatory Compliance
 - Risk management
 - Social Services
 - Staffing
- Other Items for committee review
- _____
- _____

MINUTES

Attendees:

Administrator _____ DON _____ Medical Dir. _____

_____	_____	_____
_____	_____	_____
_____	_____	_____

Minutes Submitted By: _____ Date: _____

MONTHLY RM/QI COMMITTEE MEETING

Agenda Item	Discussion	Committee Recommendation	Accountable Person Timeline
Activities Activities programming Resident Council Action plan(s) as indicated			
Admissions & Marketing Census statistics Marketing plan Action plan(s) as indicated			
Business Office Resident Trust/Petty Cash Medicare Denials/ADRs Demand Letters Action plan(s) as indicated			
Clinical Care Measures Infection Control Pharmacy Reports Action plan(s) as indicated			
Education Orientation Required Annual Education Additional Education Action plan(s) as indicated			

MONTHLY RM/QI COMMITTEE MEETING

Agenda Item	Discussion	Committee Recommendation	Accountable Person Timeline
Health Information Quantitative Audits Qualitative audits Medical Records Request Action plan(s) as indicated			
Maintenance Maintenance Reviews Line Safety Safety Committee Action plan(s) as indicated			
MDS Medicare Statistics Care Plans QI/QM analysis / Sentinel Events Action plan(s) as indicated			
Medical Director P/P Reviews Quality Concerns Action plan(s) as indicated			
Nutrition Services Tray Line/Test Trays Nutrition Services Monitoring Satisfaction Surveys Dining Room Observations Action plan(s) as indicated			

MONTHLY RM/QI COMMITTEE MEETING

Agenda Item	Discussion	Committee Recommendation	Accountable Person Timeline
Regulatory Compliance Annual Survey Complaint Surveys Self Reports DCF/APS Visits Action plan(s) as indicated			
Risk Management Incident Tracking/Trending Claims Action plan(s) as indicated			
Social Services Grievances/Family Council Behavioral Management Advanced Directives D/C Planning Action plan(s) as indicated			
Staffing Position Status New Hires Agency Use Turnover Action plan(s) as indicated			

MONTHLY RM/QI COMMITTEE MEETING

Agenda Item	Discussion	Committee Recommendation	Accountable Person Timeline

Orientation Session # 2

Understanding the Clinical Product

Clinical Care Management Meetings

Policies & procedures to support these standards are available in the following manuals:

Clinical Care Management	Additional Source for Information
Daily Clinical	
Care Review	
Weekly Care Management	Clinical Programs Manual: Clinical Care Management
Monthly Care Management	
Quarterly Care Management	

Daily Clinical Review

Go to the Clinical Programs Manual and review the following:

- ✓ Clinical Care Management Program
- ✓ Daily Clinical Review Process

Daily clinical review consists of a combination of responding to information recorded on the 24 Hour Report and results of the order and chart review process. New admissions are reviewed to assure that orders have been implemented and documentation forms are completed. New or revised physician orders are reviewed for all residents/patients to verify that orders have been implemented. Changes in condition or occurrences are reviewed to verify appropriate clinical care and responses have occurred. New information and data, such as lab results or chest x-ray results are reviewed to verify physician notification and proper clinical response has occurred. Team verifies that Physician and family notification has occurred.

Therapists meet with the Unit Mangers after Daily Operations and Daily Utilization Meeting to review resident/patient changes to determine if therapy follow-up is needed.

This process is one of the most important quality checks that occur in the business operation. This process must occur every day.

Daily Clinical meeting may be combined with Daily Operations meeting on weekends. Weekend meeting attendees include, but are not limited to Weekend Manager, Nursing Supervisor and Therapist.

Orientation Session # 2

Understanding the Clinical Product

Who is involved in the Daily Clinical Meeting?

Attendees include, but are not limited to: Director of Nursing, Nursing Managers ADON/Unit Manager

Care Review

Go to the Clinical Programs Manual and review the following:

- ✓ Clinical Care Management Program
- ✓ Care Review

The “Care Review” is a resident/patient and family/responsible party meeting with members of the interdisciplinary team. The purpose is to communicate with the resident/patient and family/responsible party to align expectations of service and include them in the care planning process. This process allows the interdisciplinary team to communicate nursing and therapy goals, and expectations for discharge, as indicated. This process also allows the facility team to learn more about the resident/patient’s psychosocial needs and quality of life needs. Care Review meetings are scheduled based, in general on the discharge plan for the resident/patient. The interdisciplinary team may determine that a meeting may need to occur sooner than indicated based on the clinical condition of the resident/patient.

As standard, Care Review meetings occur:

- Within 72 hours of admission for a short-stay patient
- Within 5 days of admission for a long term resident
- Within 72 hours of discharge

Ongoing meetings will be individually scheduled with the resident/patient and family/responsible party based on their needs. For longer stay residents, a care review may coincide with or near the monthly or quarterly care management meeting.

Who is involved in Care Review?

Attendees include, but are not limited to: Resident/Patient and/or family/responsible party, Director of Nursing/Nursing, Therapy, Social Services.

Additionally, attendees may include: Administrator, MDS Coordinator, Nutrition Services Manager, and Activity Director

Care Review Documentation Tool

The Care Review form is used to document the Care Review process and outcome. The completed form is maintained in the resident/patient medical record. (*Refer to the Clinical Program Manual for Form & Instructions*). Review a sample of the form on the following pages.

Orientation Session # 2

Understanding the Clinical Product

CARE REVIEW

Resident/Patient Name: _____

NEW ADMISSION REVIEW

DATE OF REVIEW: _____

Attendees: ☐ R/P (☐ Declined) ☐ Administrator ☐ Care Manager ☐ Social Services ☐ Nutrition Services
☐ Family (☐ Declined) ☐ DNS ☐ MDS Coordinator ☐ Therapy ☐ Other: _____

☐ Reviewed hospital discharge planner and/or attending physician expectation for length of stay: _____

☐ Reviewed resident/patient and family expectation for discharge outcome and length of stay: _____

☐ Reviewed initial assessment outcomes and estimated length of stay: _____

The following information was reviewed with the patient and family:

- | | |
|---|---|
| ☐ Team names and contact information | ☐ Physician names and contacts; visit schedule |
| ☐ Ongoing Care Management Summary | ☐ Pre-discharge education and planning |
| ☐ Review payor source questions with BOM | ☐ Discuss home evaluation process |
| | ☐ Verify needs are met |
| | ☐ Advance Directives |

Notes: _____

Signature & Title: _____

Signature of Resident/Patient or Family: _____

PROGRESS REVIEW

DATE OF REVIEW: _____

Attendees: ☐ R/P (☐ Declined) ☐ Administrator ☐ Care Manager ☐ Social Services ☐ Nutrition Services
☐ Family (☐ Declined) ☐ DNS ☐ MDS Coordinator ☐ Therapy ☐ Other: _____

☐ Reviewed progress towards goals and variances: _____

☐ Reviewed discharge expectations and anticipated date: _____

The following information was reviewed with the patient and family:

- ☐ Ongoing Care Management Summary ☐ Pre-discharge education and planning ☐ Verify needs are met ☐ Advance Directives

Notes: _____

Signature & Title: _____

Signature of Resident/Patient or Family: _____

Orientation Session # 2

Understanding the Clinical Product

CARE REVIEW

Resident/Patient Name: _____

PROGRESS REVIEW		DATE OF REVIEW: _____
Attendees: ☐ R/P (☐ Declined) ☐ Administrator ☐ Care Manager ☐ Social Services ☐ Nutrition Services	☐ Family (☐ Declined) ☐ DNS ☐ MDS Coordinator ☐ Therapy ☐ Other: _____	
☐ Reviewed progress towards goals and variances: _____ _____		
☐ Reviewed discharge expectations and anticipated date: _____ _____		
The following information was reviewed with the patient and family:		
☐ Ongoing Care Management Summary ☐ Pre-discharge education and planning ☐ Verify needs are met		
☐ Community re-entry training ☐ Home evaluation ☐ Therapy notice of discontinuation ☐ Advance Directives		
Notes: _____ _____ _____		
Signature & Title: _____	Signature of Resident/ Patient or Family: _____	

PROGRESS REVIEW		DATE OF REVIEW: _____
Attendees: ☐ R/P (☐ Declined) ☐ Administrator ☐ Care Manager ☐ Social Services ☐ Nutrition Services	☐ Family (☐ Declined) ☐ DNS ☐ MDS Coordinator ☐ Therapy ☐ Other: _____	
☐ Reviewed progress towards goals and variances: _____ _____		
☐ Reviewed discharge expectations and anticipated date: _____ _____		
The following information was reviewed with the patient and family:		
☐ Ongoing Care Management Summary ☐ Pre-discharge education and planning ☐ Verify needs are met		
☐ Community re-entry training ☐ Home evaluation ☐ Therapy notice of discontinuation ☐ Advance Directives		
Notes: _____ _____ _____		
Signature & Title: _____	Signature of Resident/ Patient or Family: _____	

CARE REVIEW

Orientation Session # 2

Understanding the Clinical Product

Weekly Care Management

Go to the Clinical Programs Manual and review the following:

- ✓ Clinical Care Management Program
- ✓ Weekly Care Management

For some residents /patients, a weekly review of their clinical condition and progress towards goals is necessary. **The Weekly Care Management meeting may be combined with the Weekly Utilization Meeting.**

Weekly Care Management is conducted for:

- ✓ A resident/patient with a discharge plan to home
- ✓ A new admission for the first 4 weeks
- ✓ A resident/patient with outcome variance until that variance is resolved

What is an Outcome Variance?

An outcome variance is any incident or change in clinical condition that affects the resident/patient's overall well being, or ability to attain goals or maintain their highest practicable level of function. It could also include a high risk treatment that may have the ability to create an outcome variance, if not monitored and managed. Examples of outcome variances may include, but are not limited to:

- | | |
|--|---------------------|
| ✓ ADL decline | ✓ New restraints |
| ✓ Changes in bowel/bladder incontinence | ✓ Pressure ulcer |
| ✓ Changes in mood/behavior | ✓ Uncontrolled pain |
| ✓ Follow up post fall | ✓ Unstable diabetic |
| ✓ Infections | ✓ Weight loss |
| ✓ Medication requiring close monitoring (i.e., Coumadin) | |

Who is involved in the Weekly Care Management Meeting?

Attendees include, but are not limited to: Director of Nursing, Unit Managers, Director of Rehab (for resident/patient on caseload), MDS Coordinator, Social Services, Nutrition Services, and Activity Director

What documentation needs to be completed during the meeting?

- Document the meeting on the "Care Management Summary" form (*Refer to Clinical Program Manual for Form & Instructions*); Review a sample of the form located after Quarterly Care Mgmt section.
- Update the Care Plan
- Update the Resident Information Sheet (care cards)

Orientation Session # 2

Understanding the Clinical Product

Monthly Care Management Meeting

Go to the Clinical Programs Manual and review the following:

- ✓ Clinical Care Management Program
- ✓ Monthly Care Management

This meeting time is to review residents/patients who may not require weekly review, but require a more frequent review than quarterly. Interdisciplinary team members review residents/patients who are receiving ongoing care and services to determine progress towards goals and to review if changes to interventions are needed. Communication with the resident/patient and family/responsible party is completed through the “Care Review” process.

A facility may choose to schedule the quarterly MDS and CAAs completion for these residents/patients at this time.

Examples of resident/patient clinical conditions that may require a monthly care meeting include, but are not limited to:

- | | |
|---------------------------------------|---------------------------------|
| ✓ Alzheimer’s /Dementia | ✓ Enteral / Parenteral feedings |
| ✓ Bowel & Bladder Continence programs | ✓ Psychiatric treatment |
| ✓ Chemotherapy | ✓ Psychoactive medication |
| ✓ Diabetic education | ✓ Restraints |
| ✓ Dialysis | ✓ Therapy services |

Who is involved in the Monthly Care Management Meeting?

Attendees include, but are not limited to: Director of Nursing, Unit Managers, Director of Rehab (for resident/patient on caseload), MDS Coordinator, Social Services, Nutrition Services, and Activity Director

What documentation is completed during the meeting?

- Document this meeting on the “Care Management Summary” form. (Review a sample of the form located after Quarterly Care Mgmt section)
- Update the care plan
- Update the Resident Information Sheet (care cards)
- MDS & CAAs as indicated

Orientation Session # 2

Understanding the Clinical Product

Quarterly Care Management

Go to the Clinical Programs Manual and review the following:

- ✓ Clinical Care Management Program
- ✓ Quarterly Care Management

The Quarterly Care Management process is also known as the “Care Plan” meeting. Regulatory requirements state that all residents in long term care, skilled nursing facility must have, at minimum, a quarterly assessment completed. Many residents/patients admitted into a skilled nursing facility have clinical conditions that require a weekly or monthly clinical care management process, but there may be long term, clinically stable residents who may not have had a clinical need requiring them to be reviewed more frequently.

This process assures that the Interdisciplinary team members review residents/patients have not been evaluated by the interdisciplinary team in the previous two months. The team meets to review resident/patient condition and to review care and services being provided and to determine progress towards goals and if changes to interventions are needed. This is also a time to assure that ongoing assessments to complete the MDS, CAAs and care plan are completed as required.

Who is involved in the Monthly Care Management Meeting?

Attendees include, but are not limited to: Director of Nursing, Unit Managers, Director of Rehab (for resident/patient on caseload), MDS Coordinator, Social Services, Nutrition Services, and Activity Director

What documentation should occur during the meeting?

- Document this meeting on the “Care Management Summary” form. (Review a sample of the form on the following pages)
- Update the Care Plan
- Update the Resident Information Sheet (care card)
- Quarterly MDS & care plans as indicated

Activity

- ✓ Complete the “**Worksheet: Operations & Clinical Care Management**”
- ✓ An “**Example of Patient Management**” can be found at the end of this session to provide additional guidance based on a fictitious patient from Admission to Discharge.

Orientation Session # 2

Understanding the Clinical Product

CARE MANAGEMENT SUMMARY

Resident/Patient Name: _____ Today's Date: _____

Reason for Meeting: ☐ Admission ☐ Scheduled Care Management ☐ Change in Condition ☐ Other: _____

Care Plan Focus	N/A	Results of Team Review			*Briefly describe identified decline, new risk factors and changes in interventions
		Progress Towards Goal	Risk Factors	Interventions	
Pain Management		☐ Pain controlled ☐ Pain not controlled*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Prevention of Skin Impairment & Wounds		☐ No skin issues ☐ Skin impairment or new wound since last review*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Mood/Behavior Prevention		☐ Improving ☐ Maintaining ☐ Declining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Improve or Maintain Nutrition/Hydration/ Address Weight Loss Oral/Dental Health		☐ Progressing ☐ Maintaining ☐ Declining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Safety/Fall & Injury Prevention/ History of Falls/ Fall Risk		☐ No falls or injury ☐ Fall or injury since last review	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Safety/Smoking		☐ Smoking safely ☐ Does not smoke safely*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Safety/Elopement Prevention		☐ No elopement ☐ Elopement since last review*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Incontinence Prevention/ & Management/ Catheter Use/ Diagnosis		☐ Improving ☐ Maintaining ☐ Declining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Therapy PT / OT / SLP (circle)		☐ Progressing ☐ Maintaining ☐ Declining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Restorative (describe) _____ _____		☐ Progressing ☐ Maintaining ☐ Declining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	
Side Rail & Restraint Reduction		☐ Progressing towards reduction ☐ Maintaining*	☐ No new risk factors ☐ New risk factors*	☐ No changes ☐ Changes*	

PS23-11-2 (Rev. 9/10) (R1033), Des Moines, IA 50306 (503) 247-2343 PRINTED IN USA.

CARE MANAGEMENT SUMMARY

Orientation Session # 2

Understanding the Clinical Product

CARE MANAGEMENT SUMMARY

Focus	N/A	Briefly describe status, factors contributing to inability to achieve goals
New Orders Since Last Review (e.g., labs, med changed, f/u appts)		
Treatments & Services (IV therapy, G-tube, respiratory care, dialysis, specialty matters, etc.)		
Prior Level of Function		
Progress Towards Resident/Patient Goals		
Barriers to Achieving Goals		
Discharge Goal		Resident/Patient & Family Communication
Goal at last review: _____ Goal after this review: _____ ☐ Home to live independently ☐ Home with assistance Describe assistance: _____ ☐ Assisted Living ☐ Long Term Care Unit Describe equipment, training or special needs for a safe discharge: _____		Review documentation from the most recent "Care Review" meeting. Next "Care Review" scheduled for: _____ Resident/Family to be contacted by: _____ Team members to be present at next meeting: _____ Notes: _____
Team Members Signatures & Title		
_____ _____ _____ _____ _____ _____		
DNS/Designee Signature indicates knowledge of the care plan and that it is appropriate for the resident/patient. ☐ Review of Advance Directives Log		
Signature & Title of Person Completing the Summary:		Date:

CARE MANAGEMENT SUMMARY

Worksheet: Operations & Clinical Care Management

To complete this worksheet, you'll need access to the following:

- Glossary: Operations & Clinical Care Management
- Administrative Manual
- Business Office Manual
- Clinical Reimbursement Manual
- Credit & Collections Manual
- Clinical Programs Manual

Complete the following questions

1. True or False "The Daily Clinical Review meeting is required every day?"
2. Select the definition that best describes "Care Review"
 - a. The meeting to review key clinical data from the last 24 hours
 - b. A resident/patient and family meeting with members of the interdisciplinary team
 - c. A meeting for the interdisciplinary team to evaluate progress towards goals and revise the care plan
 - d. A meeting to review ancillary services
3. True or False "Weekly Utilization Review" is a different name, but includes the same process and information as the "Weekly Medicare Meeting"
4. The "Care Plan" meeting is a traditional term, used often to describe the quarterly meeting required to be completed by federal regulation. This regulatory requirement can be met through which of the following meetings:
 - a. Daily Clinical
 - b. Monthly Care Management
 - c. Care Review
 - d. Quarterly Care Management
 - e. B & D

Review the procedure "Weekly Medicare Meeting" in the Clinical Reimbursement Manual and answer the following questions:

5. All of the following occur in the Weekly Medicare Meeting EXCEPT:
 - a. Ongoing identification of skilled service and evaluation of clinical documentation related to the service
 - b. Monitoring medical reviews for accuracy and timeliness
 - c. Calling to arrange discharge plans during the meeting
 - d. Tracking of ancillary services
6. True or False The Weekly Utilization Meeting is a good time to verify that required certifications such as 700/701 forms and physicians orders are completed.

Worksheet: Operations & Clinical Care Management

Review the procedure “Risk Management/Quality Improvement” in the Administrative Manual and answer the following questions.

7. All of the following indicators must be addressed in RM/QI, EXCEPT for which one?
 - a. Grievance reports
 - b. Clinical outcomes
 - c. Outstanding regulatory citations
 - d. Individual teammate issues
8. True or False Quality improvement is best achieved when just 1-2 members of the committee work on the issues.

Review the procedure “Daily Clinical Review Process” and respond to the following questions.

9. Shift-to-Shift report occurs between the off-going nurse and on-coming nurse and should include the following:
 - a. Walking rounds with verbal report of residents/patients
 - b. Review of MAR/TARs and new or changed orders
 - c. Review of residents/patients with changes in condition
 - d. Documentation of changes on 24 hour report
 - e. All of the above

Practice

Review the Care Management Meeting process. Obtain a sample tool of the Care Management Meeting Agenda and complete the following:

Mrs. Turner is a 72 year old woman admitted last week post CVA and has recently been diagnosed with diabetes. She fell last night, but was not injured. Her discharge plan is to go home with her daughter.

10. What would be the reason for reviewing Mrs. Turner in a Care Management meeting?
11. How frequently will the team be reviewing Mrs. Turner?
12. Does she have an outcome variance?
13. What type of documentation should be present in her medical record?

Clinical Indicators

1. Obtain the Clinical Indicator Report & definitions from the Portal
2. 4 of the indicators have Alert Points. Complete this section, looking back for a total of 3 weeks.

Clinical Indicators & Alert Points	Current Results	If Results are Over the Alert Point, Enter an "X"		
		This week	Last Week	2 Weeks Ago
# of total pressure ulcers that are in-house acquired (IHA) Alert Point: 3%				
# of residents with unplanned weight loss Alert Point: 3%				
# of residents with physical restraints that are not side rails Alert Point: 7%				
# of Falls Alert Point 4%				

Additional clinical data is collected on this report, but does not have an alert point associated with it. Complete the following questions to learn more about the clinical picture of your facility

Additional Clinical Data	Answer
How many residents went back to the hospital unplanned this week?	
In general, does the nursing process involve communication with or assessment by an RN prior to a resident returning to the hospital?	
In the past 3 weeks, if there are falls that occurred, did injuries occur?	
How many of those falls resulted in a significant injury?	
Where there any elopements that occurred in the past 3 weeks? If so, is there an appropriate safety plan in place for the resident today?	
What is the percentage of residents currently using side rails in this facility? (total # of residents using side rails divided by total census)	
What is the percentage of residents with chronic and/or acute symptomatic UTIs? (total # of residents meeting UTI definition in clinical indicator/care measure divided by total census)	

To complete this section, take a tour of the facility with your Director of Nursing to determine the additional clinical services that the nursing staff is providing to residents/patients

Service	# of residents/patients	% of Census
# Post Op		
Wound Care (including all wounds, wound vacs)		
IV Therapy (including intermittent & continuous)		
Enteral Feedings (including bolus & continuous)		
# Oxygen		
# Trachs		
# Dialysis	Hemo _____ Parenteral _____	
# Continuous Passive Motion (CPM)		
# Active infections being treated		

Identifying Trends

As you review the clinical data collected, look for trends in data that will help you identify the high risk areas in your facility.

For example, if the Clinical Indicators show that your facility is above the alert point for IHA pressure ulcers and the QI/QMs show that your facility is over the state and federal benchmark for pressure ulcer. This data validates each other and is likely a high risk area that you'll need to focus on.

Complete the chart below. Enter an "X" in any column that has a demonstrated outcome that is a variance of the expected level for that indicator. Some of the clinical products may not have a direct correlation to each measurement provided. The more areas marked for each clinical product, the greater the risk that could impact resident/patient outcomes or regulatory outcomes.

Discuss your results with your Director of Nursing to develop quality improvement plans to address these issues.

The Clinical Product in this facility includes care and services to address the following issues	Enter an "X" in any column that has a demonstrated outcome that is a variance to the expected level for that indicator		
	Clinical Indicator	QI/QM	Recent Regulatory Citation(s)
Pressure Ulcers (either IHA or admitted with)			
Falls			
Weight Loss			
Restraints			
Elopement			
Infections (includes UTIs and other infections)			
Pain	N/A		
Behaviors (includes depression)	N/A		
Loss of ability to complete ADL's (includes mobility, toileting,	N/A		

Risk Identification

Review the following information with your Director of Nursing. This information can provide additional insight into risks and trends in this facility.

Open Claims		Recent Reportable Events	
Recently Closed Claims		Current RM Action Items	

Example of Patient Management

Mr. Jim Granger was admitted Tuesday at 6:00pm. He is 78 years old and is a s/p hip replacement 5 days ago. He has a history of a myocardial infarction 5 years ago. Since his MI, has had atrial fibrillation, managed with digoxin. He smoked for 60 years before quitting after his heart attack. Post surgery he developed bronchitis and was started on oral antibiotics. He has a discharge plan to home with his wife. Current medications include Digoxin, Coumadin, Cipro

Timeline	What the Clinical Team Does
1/8/08 Tuesday evening	Admission: Admitting nurse completes admission documentation and receives orders for medication from the physician. Asks MD about frequency of PT/INR monitoring. MD orders a PT/INR to be done weekly, starting tomorrow. MD requests to be notified if INR is 3.0 or greater.
1/9/08 Wednesday	Daily Clinical Meeting: DON and Unit Manager conduct a Concurrent review and make sure all orders have been implemented. Note he is taking Coumadin and calls Pharmacy to review other meds for risk of potentiation. Note all admission documentation has been done and verifies therapy is aware of admission. Verifies lab will draw PT/INR today. Unit Manager makes a note to follow up on lab result this afternoon. Daily Operations Meeting: DON informs team of new admission Twenty Four Hour Report: Unit Manager notes the INR was 3.0 and physician notified. Informs MD that Pharmacist recommends discontinuing the Cipro due to potentiation of Coumadin. Coumadin order changed. MD decides to let patient finish last 2 days of Cipro. Next PT/INR ordered for 1/12/08. Care Review: Social Services talks to Mr. Granger's wife to set up a Care Review meeting for 1/11/08. Notifies team of date and time.
1/10/08 Thursday	Daily Clinical Meeting: DON verifies with Unit Manager that the new Coumadin order was received and is being administered and no bleeding is noted. Decide to schedule him for the Weekly Care Management meeting tomorrow.
1/11/08 Friday	Care Review: Pt, wife, DON, Therapist and Social Services meet to review expectations and discharge plan. Weekly Care Management Meeting & Weekly Utilization Meeting: Clinical team reviews Mr. Granger, verifies care plan has been implemented and discuss concerns about potential risk for bleeding due to receiving Cipro with Coumadin. Verifies CNAs know about risk for bleeding. Order for labs verified for tomorrow. Review of progress in therapy and current RUG level.
1/12/08 Saturday	Modified Clinical Meeting: Unit Manager reviews all labs ordered for today and confirms they are completed. Notes PT/INR is 2.8 and verifies this is recorded in the patient's medical record. MD visits facility today and notes labs. Orders PT/INR to be done weekly on Tuesdays.
1/14/08 Monday	Daily Clinical Meeting: DON & Unit Manager conduct Concurrent review, noting all new orders from the weekend.

Timeline	What the Clinical Team Does
1/15/08 Tuesday	Daily Clinical Meeting: DON & Unit Manager review labs ordered for the day and confirm PT/INR is scheduled for Mr. Granger. Weekly Care Management Meeting & Weekly Utilization Meeting: Clinical team reviews Mr. Granger and determines that patient is meeting the expected outcomes. Verify lab results drawn this morning and 2.5. Cipro has been stopped. Patient is tolerating therapy well and discharge plan is tentatively set for 2/5/08. Will plan to review next Tuesday. Progress in therapy reviewed and RUG level confirmed.
1/22/08 Tuesday	Daily Clinical Meeting: DON & Unit Manager review labs ordered for the day and confirm PT/INR is scheduled for Mr. Granger. Weekly Care Management Meeting: Clinical team reviews Mr. Granger and determines that patient is meeting the expected outcomes. Verify lab results drawn this morning and 2.6. Patient is tolerating therapy well and discharge plan is still likely for 2/5/08. Will plan to review next Tuesday. Care Review: Patient and wife met with Therapy and Social Services today to review home modifications and to do some initial transfer and gait education and training.
1/24/08 Thursday	Daily Clinical Meeting: Mr. Granger was found on the floor by his bed last night. Stated he “missed” the bed when he was trying to sit down. No apparent injury, but bruise noted on coccyx area. Physician and wife notified. DON & Unit Manager verify therapy is aware of fall and requests evaluation for ability to attend therapy today. Post Fall Action Team Review completed. Weekly Utilization Meeting: Reviewed progress towards therapy and RUG level.
1/29/08 Tuesday	Daily Clinical Meeting: DON & Unit Manager review labs ordered for the day and confirm PT/INR is scheduled for Mr. Granger. Weekly Care Management Meeting: Clinical team reviews Mr. Granger and determines that patient is meeting the expected outcomes despite his fall last week. Bruise over coccyx is the same size as day after injury. Verify lab results drawn this morning. INR is 2.8. Patient is tolerating therapy well and discharge plan is still set for 2/5/08. Care Review: Since discharge seems to be set for next week, Social Services schedule a meeting with the patient, wife and clinical team to review the discharge plan. Meeting scheduled for 2/1/08
1/31/08	Weekly Utilization Meeting: Team meets to review progress towards therapy goals and readiness for discharge on 2/5/08. Determine that therapy and nursing goals are on track.
2/1/08 Friday	Care Review: Interdisciplinary team meets with the patient and family to review plans for discharge to home have been completed. Verified knowledge of home safety including transfers and gait.
2/5/08 Tuesday	Discharged to Home

NOTES

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Introduction

For this session, you'll focus on the systems & processes for managing financial aspects of business operation. As part of this session, you'll learn about:

- Financial services & accountants
- Close schedules
- Financial reports
- Budgets
- Accounts payable management
- Petty cash
- Expense reports
- Expense Tracking System

To accomplish this session, you'll need the following resources and tools:

- Portal
- LaVie Administrative Services accounting staff

Learning activities have been provided to assist you in learning the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the Regional Vice President (RVP) and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

What to Do to Complete this Session

- ✓ Meet staff accountants and specialists from the management company
- ✓ Review current financial reports & labor reports
- ✓ Review current budget
- ✓ Learn how to submit a capital expenditure request
- ✓ Learn how to access and use the **A/P Select** system
- ✓ Learn how to use the **Expense Tracking System**

When Sessions are Completed

- ✓ Review completed sessions with the Regional Vice President (RVP)
- ✓ Complete “**Session #3**” of the **Orientation Checklist**

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Financial Support Team & Services

As the Administrator, you have a number of resources and support services available to you to assist in understanding facility business operations and finances.

LaVie Administrative Services provides accounting services to your facility. They provide complete back office support and are your first point of contact about financial matters.

Your management company has financial specialists that will review financial results with you and assist you in answering questions related to financial matters. It's important to communicate regularly, especially during month end close, to verify information is accurate.

Take the time to get to know who the accountants and financial specialists are for your facility.

Close Schedules

The close schedule is managed as follows:

Business Day #	Activities
1-3	• Payroll entries loaded by accounting • AP batches finalized by facility
1-4	• AR closed by facility Business Office Manager (BOM)
4	• AP accruals sent to accounting • Note: Accruals can be sent throughout the close process
5-6	• Detail Trial Balance posted to the Portal • Facility staff compares spend down sheets to DTTB (Expense Tracking System) • ADM prepares list of items that need to be adjusted & e-mails this list to accountant at LaVie Administrative Services • E-mail only during these two days
7-10	• Work with accountant to clean up remaining issues on financials • E-mail highly recommended
12	• Financials final & posted to the Portal
13-20 (As defined by management company)	• Operational conference calls with management specialists

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Financial Reports

There are a number of financial reports available. Reports range from a higher level “overview” to a detailed account. Your management specialist will review these reports with you in further detail.

Summary Statement of Operations

This is a “high-level” summary. This report shows areas that need to be reviewed in greater detail. It’s a snapshot of the facility financial performance in the current month and year-to-date. This report includes a complete P&L statement and information on census vs. budget

Detailed Statement of Operations

This report shows an account detail within each department and helps determine which accounts need to be reviewed in greater detail. Information includes census detail by payer type, current month, year-to-date and 3-month trend information as well as a complete P&L statement

General Ledger Detail (Detail Trial Balance)

This report shows detail within an account and allows the ADM to determine what needs to be done to correct a balance in an account. This report shows each journal entry within each account for the current month and the source of each journal entry.

Labor Reports

These are the labor reports that you will use and review routinely.

Key Factor Labor Report

Labor data is entered into this system daily (usually by the Business Office Manager). Reports are available daily on the portal.

“10 Pay Period Trend” Report

This is a trending of labor by each department per pay period

Staffing Report Extended Version

This is a report for the current pay period with detailed information on each department

Additional Reports Available (see handout on following page)

Additional reports are available on the portal to assist in managing labor. Examples of reports available include, but are not limited to: overtime, “missed punch” reports. These reports can provide you with data to assist in managing and resolving these issues timely.

Communicate any changes to your staff accountant via e-mail and maintain notes of important items that you discover so that you are prepared to discuss financials with your management specialist once a month.

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

PeopleNet Explorer Report Wizard

Page 1 of 1

HOME		PROFILE LOG OFF		PeopleNet.
SUMMARIES		HELP		
aBACK				

Select the desired report:

Maintain Existing Scheduled Reports	
<u>Report Name</u>	<u>Description</u>
Labor Distribution Reports	
1 Bi-Weekly by Department, Employee	Bi-weekly report sub totaled by Department Sorted by department (grouping), total hours per day for each employee
2 By Department (Grouping), Employee	Sorted by department, total hours per day for each employee
3 By Department, Employee	Sorted by department, shift, total hours per day for each employee
4 By Department, Shift, Employee	Sorted by department total hours
5 Hours Only	Pay - Daily, Weekly and Month to Date
6 Pay - Daily, Weekly and Month to Date Dollars by Department	Dollars by Dept
7 Payroll Hours and Dollars	Pay rate - per department total hours and dollars
8 Payroll Hours and Dollars - by Agency	Pay rate - per agency by department total hours and dollars
9 Payroll Hours and Dollars - by Clock Site	For multiple clock sites, gives hours and dollars by department
Registers	
1 Bi-Weekly Hours Only w/ Dollar Adjustments	Bi-Weekly Hours Only w/ Dollar Adjustments
2 Hours Only	Total hours for each employee
3 Payroll	Pay rate - per department hours and dollars per employee
4 Payroll - by Agency	Pay rate - per agency by department hours and dollars per employee
5 Payroll - by Clock/Site	For multiple sites, gives hours and dollars by employee by department
6 Payroll by employee, department with no subtotals	Payroll by employee, department with no subtotals
7 Total Hours ONLY summary by Employee	Total Hours ONLY summary by Employee
Punch Detail (Timecards for All Employees)	
1 Actual vs Rounded Time	Shows actual and rounded punch times as well as the variance
2 All Days - By Department, Employee	Punches sorted by department, by employee
3 All Days - By Department, Employee, Shift	Punches sorted by department, by employee, by shift
4 All Days - By Department, Shift, Employee	Punches sorted by department, by shift, by employee
5 All Days - By Employee	All employee punches
6 Approval Detail	All employee approvals
7 Bi-Weekly Timecard Report	Bi-Weekly Timecard Report

<https://www.mypeoplenet.com/PeopleNet/ReportWizard.cfm?hGUID={F8E31B3A-5840-...>

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Budgets

The budget process takes place from September – November of each year. As the facility Administrator, you are responsible for ensuring that your budget is accurate. Your accountants and management specialists will assist you in preparing the budget in an Excel file.

Following is an **example** of a budget timeline.

Timeline	Actions
August 31 st	• 1st run of budget posted to the portal • Census/Revenue based on facility trend and rates • Expenses based on facility trend (PPD basis) • FTE's based on facility staffing • Nursing based on state requirements or facility trend
September 15 th	• ADM sends list of budget changes to operations management specialist • Focus changes on census, FTE's and wage rates
September 30 th	• 2nd run of budget posted to the portal with changes reflected from 1st run • Review for other items that need to be changed • Discuss special request with the operations management specialist (e.g., adding an FTE, special projects, etc)
October 15 th	• ADM sends change request
End of October	• Meet with management specialists and finance group to finalize budgets
December 1st	• Final budgets posted to the portal

Budgets can be accessed on the Portal. Click on “Apps” and Click open “Budgets”

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Accounts Payable Management

Managing accounts payable includes effective management of the following processes:

- Vendor terms as cash model
- Invoice submission/processing
- Payment selection process
- Critical vendor management
- Petty cash
- Expense reports

Vendor Terms as a Cash Model

The purpose of establishing vendor terms as a cash model is to maintain days in A/P at a level consistent with days in A/R. Each vendor is assigned a vendor type (e.g., food, utility, medical supplies, lawn etc.). Standard payment terms are applied to each vendor type. Standard payment terms are used to determine the facility's cash available for A/P payments each week.

Vendor Type Terms	Description
Auto Pay	Garnishments, petty cash, employee reimbursement, resident funds, promissory notes, settlements
0 Days	Utility, licenses, Medical Director
30 Days	Food, Hiring adds, Medical Director, Storage, Lawn
60 Days	Dietitian, Beautician, Lab, X-Ray, Oxygen, Physicians (Non-Medical Director), Transport
90 Days	All others unless contract specifies differently

On the portal there is a list of Vendor types and when they get paid.

Invoice Submission/Processing

Invoice Review/Approval

All invoices should be billed to the facility and not to the management consulting company. If a vendor bills a facility incorrectly, immediately notify the vendor to correct. Your department heads should review all invoices for their respective department and verify that receipt of goods/services occurred. **As the Administrator, you are responsible for signing off on all invoice batches.**

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Expense Tracking System

The expense tracking system allows for detailed expense information to be tracked and monitored. Using this system will assist you in managing your budget. The Expense Tracking System has two parts, the **Transaction System** and the **Summary Report by Facility**.

Transaction System

This part of the application allows the department manager to load detailed information on expenses. Invoices should be logged into the system at least weekly.

This is an example of the Transaction page for tracking Dietary Food Cost.

Dietary Food Cost - 4206000000

Avg. Census field is Populated Automatically from the Key Factor System.-->

Average Census: 98

Monthly Budget: 14430.50

PPD Budget: 4.75

MTD Actual PPD: 4.72

Days in Month: 31

Facility:

Month: 7

Year: 2010

Preview

TransID	ExpenseDate	ExpenseAmount	Vendor	Whom	Reconcile	UserID	LastModifiedDate	Edit	Del
603408	7/6/2010	144.57	TGLEE	07/06/2010	☐		7/6/2010 5:50:51 PM		
603409	7/5/2010	129.05	TGLEE	07/06/2010	☐		7/6/2010 5:51:20 PM		
603399	7/5/2010	47.21	ST ARMANDS	07/06/2010	☐		7/6/2010 5:48:02 PM		
603402	7/5/2010	1293.55	US FOOD	07/06/2010	☐		7/6/2010 5:48:53 PM		
603404	7/5/2010	310.36	US FOOD	07/06/2010	☐		7/6/2010 5:49:28 PM		
603411	7/3/2010	132.17	TGLEE	07/06/2010	☐		7/6/2010 5:51:53 PM		
603397	7/1/2010	1430.12	US FOOD	07/06/2010	☐		7/6/2010 5:47:13 PM		
603405	7/1/2010	131.93	TGLEE	07/06/2010	☐		7/6/2010 5:50:05 PM		
603401	7/1/2010	83.09	ST.ARMANDS	07/06/2010	☐		7/6/2010 5:48:26 PM		
								Add New	

<>

Total Expense: \$3,702.05

Remaining Amount: \$10,728.45

Print

Save

Close

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Summary of Report by Facility

This report provides you with a summary of expenses from all departments. This report should be reviewed at least weekly. It's important to review if information is accurate on this report. Confirm that the reported census is correct. Pay attention to negative (noted by the color red) amounts under "Remaining Amount". This indicates an area that is off budget and will require evaluation.

Date: 7/8/2010

Expense Tracking System - Summary Report by Facility

Facility:

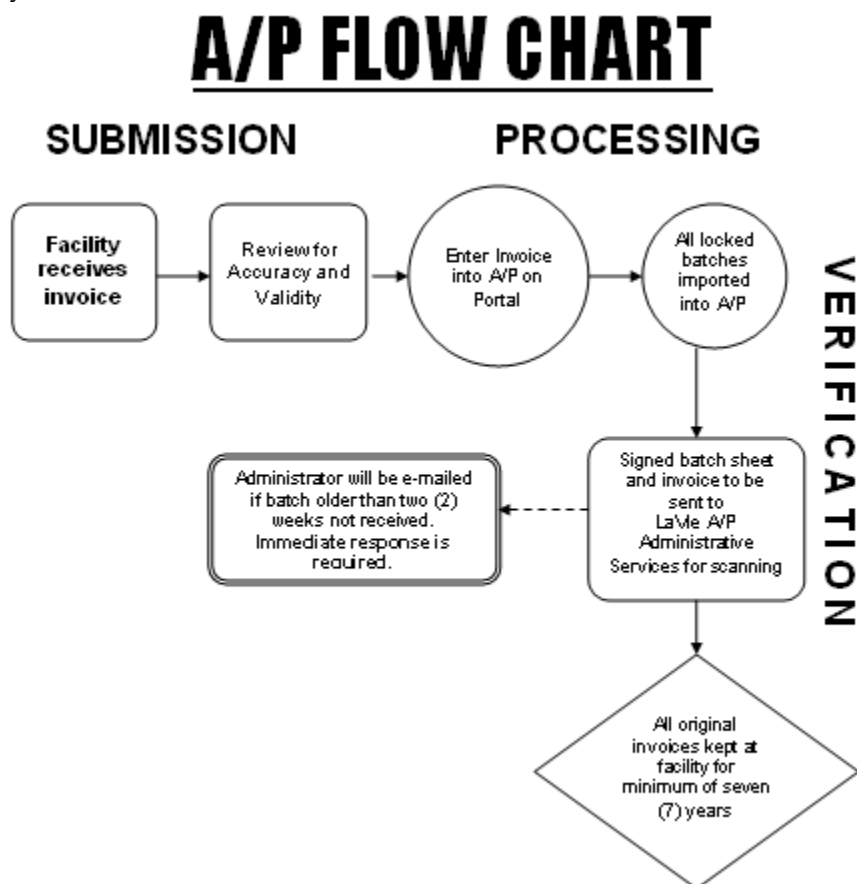
Month/Year: 6 / 2010

Expense	Average Census	PPD Budget	MTD PPD	Days In Month	Total Expense	Monthly Budget	Remaining Amount
Dietary Commercial Supplements - 4202000000	89	0.32	0.33	30	\$871.55	\$ 854.4	\$ -17.15
Dietary Food Cost - 4206000000	89	4.75	4.76	30	\$12,721.04	\$ 12682.5	\$ -38.54
Dietary Other Contracted Services - 4540000000	89	0.87	1.06	30	\$2,840.25	\$ 2322.9	\$ -517.35
Dietary Supplies - 4569000000	89	0.77	0.77	30	\$2,067.51	\$ 2055.9	\$ -11.61
Dietary Thickened Liquids - 4207000000	89	0.16	0.17	30	\$446.17	\$ 427.2	\$ -18.97
Housekeeping Other Contracted Services - 4540000000	89	0.18	0.21	30	\$564.23	\$ 480.6	\$ -83.63
Housekeeping Supplies - 4569000000	89	0.53	0.51	30	\$1,372.04	\$ 1415.1	\$ 43.06
Laundry Linens and Bedding - 4302000000	89	0.11	0.15	30	\$404.97	\$ 293.7	\$ -111.27
Laundry Supplies - 4569000000	89	0.29	0.50	30	\$1,343.10	\$ 774.3	\$ -568.8
Nursing Disposable Briefs - 4104000000	89	1.61	1.62	30	\$4,321.75	\$ 4298.7	\$ -23.05
Nursing Drugs Non Chargeable OTC - 4106000000	89	0.32	0.31	30	\$826.81	\$ 854.4	\$ 27.59
Nursing Med Sup Non Charge - 4112000000	89	1.46	1.22	30	\$3,255.36	\$ 3898.2	\$ 642.84
Nursing Supplies - 4569000000	89	0.77	0.71	30	\$1,901.71	\$ 2055.9	\$ 154.19
Plant Building Repairs and Maintenance - 4407000000	89	0.41	0.35	30	\$945.18	\$ 1094.7	\$ 149.52
Plant Equipment Repairs and Maintenance - 4404000000	89	0.70	0.71	30	\$1,907.81	\$ 1869.00	\$ -38.81
Plant Grounds Repairs and Maintenance - 4409000000	89	0.77	0.77	30	\$2,055.00	\$ 2055.9	\$ 0.90
Plant Other Contracted Services - 4540000000	89	0.75	0.90	30	\$2,398.43	\$ 2002.5	\$ -395.93
Plant Supplies - 4569000000	89	0.45	0.51	30	\$1,353.42	\$ 1201.5	\$ -151.92
Rehab Occupational Therapy Supplies - 4569000000	89	0.02	0.02	30	\$46.12	\$ 53.4	\$ 7.28

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Following is a flowchart demonstrating the submission, processing and verification status for Accounts Payable



Batching /Submission

Enter invoices into AP using the AP Portal. Keep in mind that all invoices entered on a specific date are included in one batch. All batches entered during the week should be sent and received by LaVie Administrative Services for processing by the following Tuesday.

The facility can now scan invoices. The facility keeps the original and LaVie Administrative Services receives the copies in "Ready to Scan" format.

Petty Cash should be entered into the system at least weekly.

Copies of batches should be held in a "Pending" file awaiting verification. Send batches to LaVie Administrative Services Attn: Accounts Payable.

Processing

Processing occurs when LaVie Administrative Services imports invoices into the A/P system and approves them for payment. All batches are imported the day following entry.

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Verification / Procedure

The facility can access a new A/P Select report on the portal every Tuesday at 12pm EST for Florida. At this time National does not have an AP Select. They send in check request of what needs to be paid. Check "Pending" batches against the A/P Select report. Report missing items/batches to LaVie Administrative Services as soon as possible.

Once batches have been processed, file the invoices by vendor and place batch sheets in a "Closed" file.

Payment Selection

The A/P Select program allows your facility to select invoices to be paid. Payment selection occurs once a week starting each Tuesday 12pm through Wednesday at 12pm. Invoices to be paid are "pre-selected" based on the cash model vendor terms selected for that vendor.

You have the option to "de-select" current selections and choose other invoices to be paid. Keep in mind that total invoices paid cannot be increased by facility changes. Credit invoices must be manually selected. Reasons for manual payment requests may include emergencies or capital items.

Critical Vendor Management

A critical vendor is a vendor that provides routine/monthly services to the facility for which if services should stop due to non-payment, basic facility operations, regulatory compliance and/or reputation would be at risk. Examples include Utilities, Medical Director, Ancillary Services, Lawn maintenance.

Maintain an updated **Critical Vendor List** within the facility to assist in ensuring invoice receipt, processing and payment.

Petty Cash

Petty cash must be reconciled and processed at least weekly. Keep in mind the following rules for preventing the misuse of petty cash.

- Petty cash **is not to be used** to pay service providers such as entertainers, dietitians, beauticians, plumbers etc. The facility is required to send many of these providers a 1099 form at year-end. Using petty cash by-passes the 1099 and is against IRS rules.
- Petty cash should **not be used** to pay employees, either for compensation or expense reimbursement
- Employees **are not to use** personal funds to advance petty cash when petty cash is depleted.

Orientation Session #3

Finance: Budgets, Reports & Accounts Payable

Expense Reports

Complete and submit expense reports twice a month. There is a 90 day deadline on expense reports. Any expenses reported after 90 days will not be paid. Following are some tips for ensuring timely reimbursement. Review the Expense Report policy on the Portal.

- Verify expense reports are legible
- Submit within the required timeframe
- Tape **original** receipts to an 8 ½ x 11 paper and attach behind the **original** expense report
- Keep a copy of the expense report and receipts for your files
- When reporting mileage, include total miles traveled & business purpose
- Follow expense policy guidelines for meal allowance and mileage reimbursement
- Signature of the Administrator is required. For Administrator expense reports, the Regional Vice President (RVP) should approve prior to entering it into the portal.
- Batch sheets must accompany reports and need to be filled out completely with name, vendor #, GL #
- Total each expense report separately and enter each expense report on the batch sheet
- Send expense reports directly to the address of the management consulting company (not entered at the facility anymore)

Capital Expenditures

A capital expenditure request (CER) is required to be completed and submitted for review and approval by the management consulting company.

Guidelines for CER submission are as follows:

- Items submitted via CER must have a useful life, generally 3 years or more
- Submit a CER for single items of \$500 or more
- Submit a CER for a multiple item purchase of \$1,000 or more
- Include sales tax, freight, installation and other costs related to the purchase
- Submit the CER **before** the item is purchased

Contact your operations specialist for further details & forms.

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Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Introduction

For this session, you'll focus on the systems & processes for managing reimbursement and ancillary management in your facility. As part of this session, you'll learn about:

- Clinical Reimbursement
- Medicare Management
- Daily Utilization Management
- Weekly Utilization Management
- MDS timelines
- Resource Utilization Groups (RUGs)
- MDS & Billing
- Managing a Discharge or Leave of Absence
- Medical Review Process
- Ancillary Services

To accomplish this session, you'll need the following resources and tools:

- Clinical Reimbursement Manual
- Administrative Manual
- Medicare Manual
- RAI Manual

Learning activities have been provided to assist you in learning the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the operations and reimbursement specialists from the management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

What to Do to Complete this Session

- ✓ Complete the “**Worksheet: Reimbursement Challenge**”
- ✓ Review existing vendor contracts with therapy, pharmacy, food vendors, hospice, physician advisors, medical director, specialty beds, medical supplies

When Session are Completed

- ✓ Review completed **session** with the operations and reimbursement specialist from the management company
- ✓ Complete “**Session #4**” of the **Orientation Checklist**

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Clinical Reimbursement

Attention to detail is critical for successfully managing the reimbursement process in your facility. Take the time to read, observe and ask questions about how this process is managed.

There are some key systems and processes that must be in place. In **Session #2**, you read about the **Operations & Clinical Care Management** processes that are the foundation and structure for managing your business on a daily basis. When meeting with your MDS Coordinator and Business Office Manager talk with them about these systems and processes and seek to understand how they view the role they play in making it work. These conversations are important in helping each other determine the best way to implement these systems and processes in your facility.

The primary tool used to determine reimbursement for care & services provided to the resident/patient during their stay is the Minimum Data Set (MDS). The Administrator is responsible for overseeing that this process is managed appropriately to ensure that accurate and timely information is submitted to CMS via the MDS transmission process.

Recruiting & retaining MDS staff and ensuring staff coverage for continuous MDS management is an important component to maintaining an accurate MDS that is submitted timely. The MDS Coordinator is an important member of the interdisciplinary team. The MDS Coordinator must have the knowledge and skills to provide training to team members responsible for completing portions of the MDS and the ability to coordinate frequently changing information that may impact how an MDS is completed and when it's submitted. In addition, the MDS Coordinator must also have knowledge related to the impact of appropriate management of the PPS process, MDS impact on Medicaid payment where applicable and the financial implications for facility operations.

In addition to overseeing the successful execution of the MDS process, the Administrator is also responsible for understanding the reimbursement requirements, systems and processes for Medicare Part A, B & D, the state Medicaid process, managed care contracts, insurance plans and private pay policies and procedures.

Ancillary management involves an understanding of the contracted vendor services including medical equipment suppliers and contract terms for managed care rates and reporting requirements.

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Medicare Management

Following is a brief review of important information to know when providing care and services for Medicare beneficiaries.

Medicare Part A

- 3 day qualifying stay
- Requires daily skilled services (Nursing/Therapy)
- Care must be related to hospital stay
- Must be admitted to a Medicare certified bed
- 100 days per eligibility period
- Days 1 - 20 100% covered
- Days 21 - 100 80% with 20% co-pay

Medicare Part B (also referred to as medical insurance)

- Covers therapy and supplies
- Monthly premium
- Annual deductible
- Coinsurance for most services

Medicare management begins with the admission process and requires good communication and sharing of information. The Business Office Manager will need to check and verify the following information and communicate with the interdisciplinary team:

- Check common working file for eligibility and days available.
- Obtain a copy of the Medicare card
- If resident was transferred from another SNF verify benefits
- If transferred from home, verify 30 days or less since hospital stay

There is a “Presumption of Coverage” that was passed as part of the final rule for fiscal year 2000. Under the SNF PPS, beneficiaries who are admitted (or readmitted) directly to a SNF after a qualifying hospital stay are considered to meet the level of care requirements up to and including the ARD for the 5-day assessment, when correctly assigned to one of the RUGs that is designated as representing the required level of care. A beneficiary who groups into other than one of the RUGs is not automatically classified as meeting or not meeting the SNF level of care definition. Instead, the beneficiary must receive an individual level of care determination using existing administrative criteria and procedures. This redefined the presumption of coverage and allows the skilled nursing facility to assess the resident/patient for skilled care needs up to the 5 day Assessment Reference Date (ARD).

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Daily Utilization Management

The Daily Utilization Management process was introduced in Orientation Session #2. This process allows the team time to evaluate for appropriate delivery of care and services to the resident/patient, set optimum assessment reference dates and assist the interdisciplinary team in managing the discharge process.

Members of the team involved in this process include, the Administrator, MDS Coordinator, Rehab Director or designee, Nursing, Social Services & the Business Office Manager.

Effective and efficient meetings often include focused conversations on the following topics:

- Review of all residents/patients currently receiving therapy services
- Discuss any changes in resident/patients in the past 24 hours and/or changes that may impact the plan of care
- Review new admissions and initial discharge plans
- Update of Daily Utilization log with therapy minutes and nursing services being provided
- Update from Business Office Manager on the number of skilled days available and secondary payer sources
- Hospice patients are reviewed for changes in service needs when the General In-Patient (GIP) Provision is present within the Hospice contract

Weekly Utilization Management

The Weekly Utilization Management process was also introduced in Orientation Session #2. This process allows the team to review the interdisciplinary plan of care utilizing Medicare clinical guidelines to review residents care, assist patient/resident in meeting their goals and verify documentation in the medical record reflects the care and services being provided. When providing services to Medicare beneficiaries, there are many technical issues that must be reviewed and evaluated in order to maintain integrity and follow Medicare guidelines.

Members of the team involved in this process include, the Administrator, MDS Coordinator, Rehab Director or designee, Nursing, Social Services & Nutrition Services. The Business Office Manager may also be included.

Each resident/patient is evaluated from an interdisciplinary perspective. During this meeting the following occurs:

- Projected length of stay and potential discharge plans are reviewed
- Resident/patient goals are evaluated
- Weekly Utilization Log is reviewed
- Nursing and Rehab treatments and services are reviewed for clinical appropriateness
- ADL level is reviewed for accuracy

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

- Technical requirements are discussed (e.g., SNFABN (A & B)/Notice of Medicare Non-Coverage also known as Generic notices)
- Identification of all relevant ICD 9 codes (may occur in daily meeting)
- Labs/x-rays (other vendor services) are identified/reviewed
- Ancillary services are reviewed (CME, Consolidated Billing)
- Identification of residents who exhausted benefits that need a code 22 status (Continuous Skilled Tube Feeding)
- Part B, Insurance, Medicaid therapy caseloads are reviewed for the patient's progress
- Hospice patients are reviewed for changes in service needs (e.g., GIP)

In addition to reviewing residents/patients currently in the facility, the team also conducts a 30 day look back on all Medicare discharged residents/patients to assess the need to resume skilled coverage for residents/patients that meet Medicare guidelines.

MDS Review

The following information is provided as a brief review. Review this information with your MDS Coordinator and Reimbursement specialist as needed.

PPS MDS's	OBRA Required MDS's
• 5 day • 14 day • 30 day • 60 day • 90 day • OMRA ○ SOT ○ EOT	• Admission • Quarterly • Annual • Significant Change • Discharge - Return not anticipated; Return anticipated; Death in facility • Entry ○ Admission ○ Re-entry

Selecting an Assessment Reference (ARD) Date

MDS	ARD Window	Grace Days
5 day	1 - 5	6 - 8
14 day	11 - 14	15 - 19
30 day	21 - 29	30 - 34
60 day	50 - 59	60 - 64
90 day	80 - 89	90 - 92

Additional assessments both optional and required have been added with MDS 3.0. Some examples are Medicare Short Stay Assessment, Start of Therapy, and OMRA.

*Failure to set the ARD within the reference window will result in default days

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Grace Days

The final rule (FR) for SNF PPS for fiscal year 2000 effective for service dates on or after October 1, 1999 states: **The final rule has not altered the number or use of grace days but has clarified their use and application.** The use of grace days is acceptable and permitted for patients with any condition.

Days one through five are optimal for a 5 day Assessment Reference Dates (ARD); however, days six through eight are also acceptable and for some residents, (e.g., those receiving rehabilitation services would be the most appropriate).

Resource Utilization Groups (RUGs)

RUGs determine the daily rate that the facility will receive for providing care and services to a Medicare beneficiary. Following is some information about RUGs:

- 108 data fields from the PPS MDS
- Classifies based on the RAVEN software
- Rate attached to each of the 66 RUGs
- Based on Urban or Rural and wage index
- Can change on an annual basis
- There are 66 RUGs based on clinical criteria in each of the 8 categories along with ADL and Clinical end splits

There are 8 major groups of RUGs

- | | |
|---------------------------------------|---|
| • Rehabilitation + Extensive Services | • Special Care Low |
| • Rehabilitation | • Clinically Complex |
| • Extensive Services | • Behavioral Symptoms & Cognitive Performance |
| • Special Care High | • Reduced Physical Functioning |

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

MDS & Medicaid Billing

Timely and accurate completion and submission of the MDS is important in meeting the key timeframes to receive payment for care and services provided.

- 5 day MDS Covers Days 1 - 14
- 14 day MDS Covers Days 15 - 30
- 30 day MDS Covers Days 31 - 60
- 60 day MDS Covers Days 61 - 90
- 90 day MDS Covers Days 91 - 100

There are two types of OMRA – SOT OMRA and an EOT OMRA. The SOT OMRA is an optional assessment and can be completed anytime during a Medicare Part A stay to classify a resident/patient into a Rehab group. The EOT OMRA is a required assessment that is completed when a resident/patient continues to need Part A services after the discontinuation of all therapies.

Managing a Discharge or Leave of Absence

Discharge

In order to discharge a resident/patient from Medicare, one or more of the following criteria must be met:

- Team determines that the resident/patient has met his/her goals or would not benefit from services any longer, or
- The resident/patient exhausts Medicare benefits or,
- The resident/patient meets discharge goals for discharge destination

Upon determination of discharge, a Notice of Medicare Non-Coverage form and SNFABN if applicable is given 3 days prior to discharge. The facility tracks Medicare discharges for 30 days

Leave of Absence

Any time a resident leaves the facility and does not return prior to midnight, this is considered a leave of absence. A leave of absence may be a Medical LOA or a Therapeutic LOA. In the event of a LOA, Medicare Part A days are not used. LOA days do not interrupt the Medicare assessment schedule and are not counted on the MDS when scoring the resident/patient's next assessment.

Orientation Session #4

Understanding Reimbursement, Ancillary Services & Medical Review

Medical Review Process

Fiscal Intermediaries are contracted by CMS to manage both the distribution of payment and program integrity for Medicare. These Fiscal Intermediaries are now referred to as Medicare Administration Contractor (MAC).

Medical Review is the method used by MACs to ensure Medicare program integrity and appropriate claims payment. Facilities that have been selected for focused review can have 100% of claims reviewed. Files selected for medical review are selected from the following:

- Pre Pay (ADR's)
- Random Post Pay
- Focused Medical Review
- 100% of Demand Bills
- Probe Review
- Focused CMS Reviews (RAC, CERT)

The process can consist of (1) ADR (Additional Documentation Request); (2) Re-determination; (3) Reconsideration and (4) ALJ (Administrative Law Judge) for part A and B claims of \$130 or more.

During an ADR, payment is suspended until the review is complete. A typical ADR response time for pre-pay is 30 days and post-pay is 30 days.

Reasons for denial of payment may include: timeliness of records sent, level of care not met, services statutorily excluded or determined that unreasonable or unnecessary services were provided.

Tips for a Successful Medical Review

- BOM checks the DDE daily for new ADR's and for any additional correspondences via mail or fax requesting records (i.e. CERT OPs, RACs, etc.).
- Review documentation on an ongoing basis to confirm that the medical record accurately reflects the skilled services provided
- Review the medical review request prior to record review with the reimbursement specialist, or designee, and the rehab clinical specialist to ensure all technical and clinical documentation requirements are met
- Review medical review requests and process with the reimbursement specialist. Your specialist can provide you with tools and guidelines to assist in this process. (Refer to the Medical Review grid on the Portal)
- Verify that a responsible staff member is overseeing the process to prepare the ADR package
- Send the ADR package out timely
- React to any correspondence from FI timely
- Notify all necessary parties of ADR & decision by maintaining the Medical Review Tracking System on portal, current and accurate

Hospice

Read & review the **Hospice Policies & Procedures** in the Administrative Manual. Review any hospice related questions with your reimbursement and operations specialist.

Worksheet: Reimbursement Challenge

To complete this worksheet, you'll need access to the following:

- MDS Coordinator
- Reimbursement Specialist
- Clinical Reimbursement Manual
- Administrative Manual

Instructions:

1. Review and consider the following questions.
2. Discuss questions & answers with your MDS coordinator and reimbursement specialist

Questions	Answers
1. How do resident goals and anticipated length of stay differ between Medicare Part A and Managed Care?	
2. What is the benefit of meeting weekly with the Director of Rehab to review therapy treatment plan/goals & therapy minutes for residents with managed care and Medicaid?	
3. If a resident is able to tolerate the activity, what would be the benefit of a therapy evaluation on the day of admission?	
4. If a resident is using a specialty bed due to skin integrity issues, what would be the indicators to determine when the bed is no longer necessary?	
5. What is the purpose of a therapy home evaluation?	
6. What are "grace days" and when are they appropriate to use?	
7. Who sets the Assessment Reference Date (ARD) date?	
8. Which staff members should be involved in the Daily/Weekly Utilization meeting and why are the meetings important?	
9. Describe the interdisciplinary team members who should attend the Match Meeting. How often should this meeting occur?	
10. Describe consolidated billing and letters of agreement.	
11. Describe the General In-Patient Hospice provision.	

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Orientation Session #5

Business Office Services

Introduction

For this session you'll focus on the systems & processes for managing business office services in your facility

An efficient and well-performing business office is the responsibility of the Administrator. Learning the rules, regulatory requirements & systems for billing, collecting and managing the financial aspect of your business are crucial to maintaining a well run and successful operation.

Your goals for this session are to learn and understand the policies, procedures and systems to follow for managing business office services.

Learning activities have been provided to assist you in learning the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with your specialist and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do To Complete This Session

- ✓ Complete the "**Internal Control Letter**" immediately upon hire
- ✓ Complete the "**Worksheet: Business Office Services**"
- ✓ Attend "**Credit & Collections Training**"

When Sessions are Completed

- ✓ Review completed sessions with the business office services and operations specialists from the management company
- ✓ Complete "**Session #5**" of the **Orientation Checklist**

Date: _____

Dear Sir or Madam:

As a new employee with FACILITY NAME, it is important that you understand areas related to protecting assets and internal controls. This letter will provide you with an overview of this information. Please reference your Business Office Policies & Procedures for additional information and clarification. Once reviewed, please sign. (Retain a copy for your records)

BANK ACCOUNTS

Please be aware that your facility has very specific authorized bank accounts. These accounts have been established, using the proper procedures, by the Treasury Department at LaVie Administrative Services on behalf of the Facility. The accounts that should be used for all of your routine business purposes include: 1 - Depository account, 2 - Resident Trust Fund account and 3 – Petty Cash Resident Trust Fund bank account.

It is important that you understand and inform your staff that additional bank accounts should not be opened (in the facility's name nor in an employee's name for the facility) for any reason.

OTHER FUNDS

Often times your facility will find the need to secure funds other than your routine cash receipts and Resident Trust Fund monies, e.g. Facility fundraisers for employees and residents, donations, Resident Council, etc. Under no circumstances should you or your facility establish separate bank accounts or maintain "slush" funds to manage these funds. Please refer to the Other Funds Policy for proper accounting.

MISC REVENUE (Vending Machine, Postage, Meal sales)

The facility should handle all other incomes (meal money, postage money, vending machine money) consistent with other cash received. The money should be receipted in the depository receipt book upon receipt, deposited daily and posted to the proper G/L account.

FACILITY PETTY CASH vs. RESIDENT TRUST FUND PETTY CASH

Your facility maintains these two Petty Cash funds. These funds should not be co-mingled and should be maintained in perpetual reconciliation. Refer to your policies & procedures for a definition of proper use of these funds.

RESIDENT VALUABLES

Your facility should discourage the handling of resident valuables. Under rare circumstances where a resident is admitted with valuables that need to be temporarily secured, please refer to your Resident Valuables policy & procedure for proper handling.

DEPOSITS

Accuracy of your deposits is critical. Whether you deposit into the bank or utilize the Remote Deposit Capture system, your staff is expected to ensure accuracy of all deposits by selecting the proper account or preparing the proper deposit ticket. Should an error occur and funds are deposited into the wrong account, no facility staff members should be asking the bank to correct the deposit. By doing this, you are circumventing the Internal Controls within your facility. Please refer to your policies and procedures and/or contact your consultant for assistance on correctly this proper way.

Additionally, it is expected that the facility maintain 2 separate receipt books, 1 for Depository and 1 for Resident Trust. There should never be more than 2 receipt books active. Receipts must be written in sequence and voided receipts defaced and reattached to the original receipt book.

Proper receipting is defined as having 3-copy receipt book. The original is given to the individual making payment, the 2nd copy should be attached to the Cash Receipt Listing report and the third copy should be retained in the receipt book at all times. Voided receipts must be defaced and all three copies reattached and retained in the receipt book.

Correcting deposits:

- Monies deposited into the Depository account that should have been deposited into the RTF account:
 - Contact your Administrator and specialist immediately for guidance
 - Prepare Refund request
 - Have specialist expedite request
 - Document appropriately
- Monies deposited into RTF account that should have been deposited into the Depository account:
 - Contact your Administrator and specialist immediately for guidance
 - Post deposit, prepare withdrawal, issue check to Facility Depository account and deposit into Depository account
 - Document appropriately

Note: It is not proper procedures to complete a "Request for Correction" from National Data Care to have the funds transferred between accounts.

COMMUNICATION

The Administrator is responsible to immediately report any violation of Internal Controls to the appropriate individuals. This includes, but is not limited to: theft of any kind, unreconciled Resident Trust Fund, suspicious activity, co-mingling of funds, inappropriate account adjustments, inappropriate payroll activity, inappropriate accounts payable activity, etc.

RESIDENT TRUST FUND

No Business Office Staff should be a signer on this account. All signers and Business Office staff should have reviewed and understand the Resident Trust Fund policies & procedures.

SIGNATURE CARDS

All signature cards are to be obtained through the Treasury Department at LaVie Administrative Services. (Please contact the Treasury Department for proper procedures) No facility staff should obtain or attempt to change these cards on their own. The Administrator and Director of Nursing are required to be on the signature cards. No Business Office personnel should be listed as an authorized signor on the signature cards.

Please acknowledge you have received and reviewed this by signing below.

If you need additional clarification regarding any of the above areas, please contact the RVP or RBS who can provide you with additional guidance.

Title

Print name

Signature

Date

Worksheet: Business Office Services

Complete the following sessions within the recommended timeframe. Initial when complete.
Note any questions to review with your Regional Business Specialist.

Session	Timeframe	Notes
Internal Control Letter Review, sign and return the letter to your specialist. There are several sessions within the letter that will need to be accomplished. It is important that each item be addressed thoroughly.	Immediately upon hire Initial	
Meet Business Office Manager (BOM) and Business Office Team	Within 2 days (refer to Session #1) Initial	
Meet with BOM to learn of any immediate needs for collection assistance	Within 2 days Initial	
Schedule Weekly Business Office Manager meeting Obtain guidelines for meeting from RVP	Within 7 days Initial	
Observe Medicare Match Meeting	Within 7 days Initial	
Observe Medicaid Pending Meeting (meeting time depends on volume)	Within 7-14 days Initial	
Cash Measurement System Review Report Card Trend, MTD Cash Report, Cash Reporting System and Exception Report	Within 2 weeks Initial	
Read & Review Administrator Standards	Within 2 weeks Initial	
Review Business Office (B.O.)Calendar	Within 2 weeks Initial	
Review and evaluate Business Office Improvement Plan	Within 2 weeks Initial	
Perform Accounts Receivable Review with Business Office Manager	Within 30 days Initial	
Review and evaluate Business Office Cross-training Checklist	Within 30 days Initial	

Worksheet: Business Office Services

Session	Timeframe	Notes
If BOM has been hired within 120 days, review and evaluate BOM's orientation checklist	Within 30 days Initial	
Obtain & Review Business Office Policy & Procedure Manual and Credit & Collection Manual .	Obtain upon hire. Review within 60 days Initial	
Review Bonus plan for the Business Office Manager	Within 45 days Initial	
Attend Credit & Collections Training	Next scheduled training Initial	

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Orientation Session #6

Census Development & Admission Process

Introduction

For this session, you'll focus on the systems & processes for census development & admissions in your facility. As part of this session you'll learn about the facility processes for:

- Developing a Marketing Program
- Researching Referral Sources
- Medical Model
- Census Development
- Admissions Process
- Discharge Management

To accomplish this session, you'll need the following resources and tools:

- Administrative Manual
- Access to the Portal

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with your specialist and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do To Complete this Session

- ✓ Complete the **"Worksheet: Census Development & Admission"**
- ✓ Review the facility **Sales & Marketing Plan**
- ✓ Visit local referral sources
- ✓ Observe the admission process from referral through admission
- ✓ Review the Discharge Management process with the interdisciplinary team

When Sessions are Completed

- ✓ Review completed sessions with the management company specialist
- ✓ Complete **"Session #6"** of the **Orientation Checklist**

Orientation Session #6

Census Development & Admission Process

Facility Sales & Marketing Team

Members of the facility sales and marketing team include the Administrator, Admission Specialist, and Admissions Director. The team works closely with the Director of Nursing and the facility clinical team to identify.

Developing a Marketing Program

A strategic sales & marketing plan includes the following components.

- Analysis of market needs
- Staff preparation & education to meet market needs
- Identification of key referral sources
- Developing relationships to increase market share and “same store” growth

As the Administrator, your role is to manage the team and oversee the program. Revisions to the sales and marketing plan are then made according to market changes, level of referral acuity and acute & post acute resident/patient placement needs.

Staff & Facility Preparation

Staff education and clinical competencies may need to be routinely reviewed to ensure that staff is prepared to provide care and services to patients being admitted from referral sources. In some instances, staff with different skill sets may need to be hired to support the clinical program focus.

Talk with your Director of Nursing and Admission Specialist and learn about the communication process to relay referral information to clinical staff. Clinical information about the referral, as well as equipment and supply needs are important steps in creating a smooth and successful admission.

With frequent admissions and discharges, having well organized meetings and systems becomes very important in running a successful facility. Information on meeting management can be found in the “Operations & Clinical Care Management” in Orientation Session #2.

Orientation Session #6

Census Development & Admission Process

Census Development

Managing census development is a daily part of operations. Once a sales and marketing plan is implemented, the Administrator, Admission Specialist & Admissions Director meet daily, usually after the Daily Operations Meeting to discuss and plan for:

- ✓ The number of needed referrals and admissions to exceed the discharge and census goal
- ✓ Expected discharges
- ✓ Staying focused on growing admissions identified in the plan
- ✓ How to adapt strategies to increase admissions identified in the plan

Daily Operations Meeting

During the daily operations meeting, an update from the sales and marketing team should include the following:

- ✓ Census
 - Opening census
 - Opening mix
 - Comparison to budget
- ✓ Average Length of Stay
 - Month-to-date
- ✓ Pending Referrals
 - Total referrals by diagnosis, doctor, hospital & payor
 - New referrals by diagnosis, doctor, hospital & payor
 - Conversion ratio of referrals to admissions
 - Lost business or denials and plan to resolve
- ✓ Admissions
 - Projected admissions
 - Total admissions
- ✓ Discharges
 - Scheduled discharges
 - Unscheduled discharges

Orientation Session #6

Census Development & Admission Process

Admission Process

During the admission process, the Admission Specialist and/or Admission Director collects initial clinical and billing information using the **“Pre-Admission Assessment Tool”**. This information is provided to the Admissions Director, Director of Nursing and Business Office Manager prior to the arrival of the resident/patient at the facility. During this process, it’s important to identify bed availability, clinical care needs and payment information.

Creating a smooth transition requires a thoughtful approach to anticipating what the residents/patients and family may need to hear, read or see to help them during this time. How your team presents itself during this time creates a first impression for the new resident/patient and family and sets the tone for what they can come to expect of their stay at your facility.

If you believe that your facility cannot accept a potential resident/patient referral, review with your management specialists to determine if there are additional resources available in order to be able to accommodate their needs.

Spend some time learning how the admissions process works in your facility.

Discharge Management

Managing census is probably better described as an “art” than a “science”. It requires a focus on meeting a resident/patients needs, flexibility and the ability to ask good questions of your team. If your facility has a high number of admissions and a shorter length of stay, the more challenging it may become to maintain a stable census. Following are some tips and guidelines on navigating this process:

- **Designate one team member as the Discharge Coordinator.** This allows for one person to coordinate discharge plans, establish post discharge services and communicate the expected discharge date. Increased customer satisfaction, improved discharge management and a decrease in miscommunication are often the results of this type of process.

Orientation Session #6

Census Development & Admission Process

- **Maintain a consistent schedule of meetings.** These are core components of your business operations and the method for keeping your business running smoothly. **Daily Operations** meetings provide team members with the opportunity to stay current on what is occurring with your patient population and allow team members to ask questions of each other, such as validating physician orders for treatment and how a patient is progressing towards goals. The **Care Review** meeting should occur initially within 72 hours after a patient is admitted and as needed thereafter. This meeting allows key members of the team to meet with the resident/patient and family/responsible party to discuss their expectations and to discuss what to expect from their stay.
- **Coordinate discharges to best meet the patient's needs.** If a patient is expected to need home health services, special pharmacy services or therapy services, schedule the discharge to occur on a weekday. This will ensure for a smoother and more successful transition to home for the patient and family/responsible party.
- **Understand how your therapy provider evaluates a patient's readiness to go home.** Does your therapy provider routinely conduct Home Evaluations for patients? How resident/patient readiness to go home is communicated to the resident/patient, family/responsible party and interdisciplinary team?
- **Maintain a Discharge Calendar.** Assign a designated person to record anticipated discharge dates on the calendar and provide an updated copy to the interdisciplinary team for review at the Daily Operations meeting.

Review the Discharge Management process with your interdisciplinary team.

Worksheet: Census Development & Admissions

To complete this worksheet, you'll need access to the following:

- Administrative Manual

Instructions:

1. Review and consider the following questions.
2. Discuss questions & answers with your Sales & Marketing team

Questions	Answers
1. What is the pre-assessment tool? Who completes the tool?	
2. What is the "Medical Model"?	
3. Describe the role of the Physician Advisor	
4. During the Daily Operations Meeting, what information is provided by the sales & marketing team?	
5. Who coordinates discharges in your facility?	
6. Managing census is a part of daily operations. List the primary areas to monitor.	
7. What meeting should occur within 72 hours after a resident/patient admission?	
8. What is the purpose of Care Review meetings?	
9. Describe strategies for Discharge Management	
10. Who are the members of the Sales & Marketing team?	

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Orientation Session #7

Human Resources & Staff Development

Introduction

For this session, you'll focus on the systems & processes for managing human resources and staff development in your facility. As part of this session you'll learn about the facility processes for:

- Performance Management
- Recruitment & Retention
- Regulatory Compliance
- Labor Relations & Union Avoidance
- Workers Compensation
- Training & Development

To accomplish this session, you'll need the following resources and tools:

- HR Policy & Procedure Manual
- HR Forms Manual
- Employee Handbook
- Safety & OSHA Compliance Manual
- Education Resource Manual
- Workers Compensation Manual
- Current union contract if applicable
- Access to the Portal
- Development Feedback Tool

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the Team Specialist from your management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #7

Human Resources & Staff Development

What to Do To Complete This Session

- ✓ Complete the **“Worksheet: Human Resources”**
- ✓ Review the facility **Education Calendar**
- ✓ Complete the **“Leadership in LTC”** course on the on-line university
- ✓ Review the **HR Policy & Procedure Manual & HR Forms Manual**
- ✓ Review the **Employee Handbook**
- ✓ Review the **Safety & OSHA Compliance Manual**
- ✓ Review the **Education Resource Manual**
- ✓ Review the current **union contract**, if applicable
- ✓ Review **“Development Feedback”** tool

When Sessions are Completed

- ✓ Review completed sessions with the Team Specialist from your management company
- ✓ Complete **“Session #7”** of the **Orientation Checklist**

Orientation Session #7

Human Resources & Staff Development

HR Policies & Procedures

The HR Policy & Procedure Manual and HR Forms Manual will provide you with information and guidance related to human resource management. Verify you have a manual and take the time to read the following policies & procedures:

- ✓ PTO & VHS
- ✓ Coaching
- ✓ Absenteeism
- ✓ Exempt vs. Non Exempt
- ✓ Harassment Free Workplace
- ✓ Clocking In/Out—Lunch punches
- ✓ Drug & Alcohol Free Workplace

Performance Management

As the facility leader, it's your responsibility to promote the importance of teammate performance management as an ongoing, frequent part of a manager's position. Department leaders should understand how to conduct teammate evaluations and learn how to provide regular feedback to teammate. The content of a teammate evaluation should not come as a surprise to the teammate.

Evaluations and performance assessments should be completed 90 days after a teammate is hired. This may be extended for another 30 days if needed to allow for further review. After this initial review, teammate evaluations are to be conducted yearly thereafter unless the plan requires a mid-year review. All reviews & pay adjustments must be done timely.

Coaching

Coaching is the formal process used to provide feedback and guidance towards improving teammate performance. Coaching, when done well, promotes a supportive work environment and demonstrates to staff that you are interested in their achieving a higher level of performance, not in punishing them. Coaching is not the same as disciplining. Discipline is punitive and can result in a de-moralizing affect on teammates. Coaching should be focused on addressing those issues that require improvement and what can be done to help develop the skills needed to improve performance.

Orientation Session #7

Human Resources & Staff Development

There are two coaching levels, Level 1 Coaching and Level 2 Coaching.

Level 1 Coaching

This level of coaching is for minor infractions such as attendance and work performance issues. The goal is to identify the issue and develop skills or strategies to improve.

Strategies to use when conducting this level of coaching include:

- ✓ Conduct the coaching in a private area or office
- ✓ Specifically state the problem using clear and descriptive language
- ✓ Ask for and listen to the teammate point of view
- ✓ Identify the changes that are needed
- ✓ Work together to develop a plan to correct
- ✓ Commit the plan in writing on a coaching form
- ✓ Follow up with the teammate within 30 days

Level 2 Coaching

This level of coaching is for major infractions such as theft, abuse, harassment etc. The goal is to conduct a thorough investigation and determine if employment is to be continued or terminated. The teammate may be suspended during this stage of the investigation

The processes to follow when conducting a Level 2 coaching include:

- Conduct the coaching in a private area or office
- Specifically state the problem using clear and descriptive language
- Advise the teammate that he/she is suspended until the investigation is complete
- Offer the teammate the opportunity to give a statement or provide details about what occurred
- Document the meeting using a Level 2 coaching form
- Notify your Team Specialist when a suspension occurs & prior to termination

Conducting a Level 2 Investigation

- Complete the investigation in a timely manner to avoid lost work time for the teammate
- Interview all persons involved and all witnesses. Document findings.
- Consult with your Team Specialist on any questions about the investigation process
- Do not use a Level 2 suspension as a punitive measure

Orientation Session #7

Human Resources & Staff Development

Possible Outcomes after a Level 2 Coaching

- Teammate is cleared of all allegations, returns to work and is paid for all lost time
- Teammate coaching is reduced to a Level 1 and teammate returns to work and paid for lost time in excess of 2 days
- Coaching remains at a Level 2, but teammate returns to work and given a final warning status. Review with your Team Specialist prior to discussing with teammate.
- Investigation confirms allegation and teammate is terminated. Review with your Team Specialist prior to terminating the teammate.

Additional tips to consider when coaching include, but are not limited to:

- ✓ Don't wait to utilize the coaching process when issues have gotten to the point of "you've had enough". Address issues when they are initially identified.
- ✓ Avoid completing the coaching form ahead of time. This sends the message that you have already come to the conclusion of what has occurred and you have not given the teammate the opportunity to provide their thoughts on the situation.
- ✓ Remain open to the teammate's perspective
- ✓ Avoid giving options such as "You can resign or be fired"

Orientation Session #7

Human Resources & Staff Development

Recruiting & Retaining Staff

Assembling a team with the right skills and work ethic is critical to running a successful business. In order to attract & retain staff to work in your facility, ensure that you are managing the following:

- ✓ Routinely and annually in coordination with budget preparation, evaluate wage scales and internal equity. Obtain assistance from your management company as needed.
- ✓ “Re-recruit” staff daily. Get to know your staff. Observe how supervisors interact with staff and provide feedback. Lead by example. Work your recruitment and retention plan.
- ✓ Promote in-house teammate incentive & satisfaction programs
- ✓ Recognize staff accomplishments, demonstrate respect and celebrate your staff
- ✓ Advertise and promote open positions. Promote from within when possible.
- ✓ Point of sale/applicant processing. This means providing a comfortable and inviting environment for new applicants. Example may include, but are not limited to: offering a quiet area to complete paperwork, offering coffee or something to drink and responding promptly to questions.

Orientation Session #7

Human Resources & Staff Development

Regulatory Compliance

As a healthcare employer, there are multiple regulatory agencies that provide rules and regulations that must be followed. Agencies include, but may not be limited to:

- ✓ OSHA / Occupational Safety & Health Administration
- ✓ EEOC / Equal Employment Opportunity Commission
- ✓ Department of Labor (Wage & Hour)
- ✓ OFCCP / Office of Federal Contract Compliance Programs
- ✓ CMS / Centers for Medicare & Medicaid Services
- ✓ State regulatory agencies

Your Team Specialist will review the rules and regulations with you during your orientation. If an agency contacts you via phone, mail or survey, it's very important to respond immediately to avoid penalties. Consult with your management company specialist on any inquiry from a government agency to discuss ways to respond.

Labor Relations & Union Avoidance

Developing a positive relationship with your staff can result in many benefits. Increased staff satisfaction has been shown to increase resident/patient satisfaction. Satisfied staff is more likely to continue working in your facility, thus reducing the amount of overtime and costs related to recruiting and training new staff. As a result, labor relations and union avoidance become easier to manage.

Frequent communication with staff, at all times, and all shifts will help to build those positive relationships. Listening and responding to staff help create an open and positive work environment. As a new Administrator, you have the opportunity to create a new and positive history with your staff.

If you currently have a union in your facility, read the contract and make sure you understand the terms. When contacted by a union representative, consult with the Team Specialist from the management company before responding.

Orientation Session #7

Human Resources & Staff Development

Training & Staff Development

Successful Administrators understand the importance of investing time and resources in training and developing teammates. Teach your department managers how you see and understand your business. Help them learn how to critique and analyze their departments and their outcomes and how to develop strategies to improve. This is time well spent and you will reap the benefits of your efforts. Remember, telling is not teaching. Describe, explain and demonstrate what you mean.

Your department managers, in turn, must be able to provide training and staff education to teammates on topics related to their assigned roles and responsibilities.

Meet with your Regional Quality Specialist to learn about the facility education calendar and learn about the status of meeting the mandatory education and regulatory compliance requirements. The **Education Resource Manual** provides guidance and ideas on meeting regulatory requirements, improving teammate retention and how education initiatives can support facility business growth.

Leadership and Supervisor Training

Any teammate with management or supervisory experience should receive education and training on leadership and supervisory skills. It's the responsibility of the Administrator to ensure that this training is being provided. You'll find resources for providing leadership and supervisory training from your Team Specialist or by visiting the on-line university available to you and your staff.

Utilizing the On-Line University (www.laviewuniversity.com)

An on-line university is available to all teammates (**except Rehab PRN teammates**) of the facility. There are many features and benefits that come with using this resource. Courses range from short in-services to CEU approved courses.

You and your teammates may find this a useful resource for:

- Accessing training and in-service materials
- Providing an additional option for teammates to complete regulatory requirements
- Promoting teammate directed learning
- Promoting teammate growth, both personally and professionally
- Resource for teammate requiring CEU's to renew licensure or certification

Orientation Session #7

Human Resources & Staff Development

In addition to being a resource for educational content and learning, the on-line university is also a Learning Management System (LMS). An LMS provides the Regional Quality Specialist with management tools including:

- Ability to maintain electronic, individual education files for each teammate
- Ability to upload in-services or training session taught live, and record individual attendance
- Ability to assign in-services or courses for teammates to complete
- Ability to run a variety of reports including which teammates completed an in-service or course, or which teammates haven't

The on-line university is available to all teammates 24 hours/day, 7 days/week and is web based, so it can be easily accessed from work or from home. Teammates are free to take as many in-services and courses as they wish, all at no charge to the teammate.

Note on Wage & Hour Laws: Employers are required to pay teammates for time spent during job related training & in-services. When assigning teammates to take on-line courses, make sure to schedule time to complete during working hours.

The on-line university can be a tremendous benefit to teammates, but only if they can access it easily. Check to see if your facility has a computer with internet access and a printer available for teammates to access at any time to take in-services and courses. **If your facility doesn't have a dedicated computer for training and development, develop a plan to obtain.**

Topics available online may change periodically and new courses added. Some of the current topics available include:

- *Coaching for a Better Performance (Course #1110)*
- *Creating & Maintaining a Nurturing Environment (Course #1031)*
- *Leadership in Long Term Care (Course #1093)*
- *LPN Supervisory Course (Course #1139)*

Additional Resources

Talk with your Regional Quality Specialist and Department Managers to find out what resources they are using to provide training to staff.

Orientation Session #7

Human Resources & Staff Development

Development & Feedback Process

The Development Feedback tool is an interactive process designed to assess a teammate's skills profile and career goals. The purpose of the Development Feedback tool is to provide an organized process by which a supervisor and a teammate can discuss development opportunities and then create specific action plans to address them. All supervisors are responsible for providing this tool to their teammates.

The Development Feedback tool should be completed once a year for all teammates. LaVie has designated June, July & August of each year as "Development Feedback Season". All supervisors should meet with teammates during this time to review and discuss the Development Feedback form. The Development Feedback tool is designed to promote interactive communications between the teammate and the supervisor. This tool will not be used to determine salary increases. A teammate's wage increase is determined separately based on facility budget considerations.

The ultimate goal is to proactively enhance the personal and professional development of each teammate, help him/her to be successful in and enjoy his/her existing role and to assist each teammate to achieve his/her long-range career goals.

All supervisors are responsible for completing development feedback with their teammates within 90 days of hire and annually. Additional forms and processes used to evaluate a specific teammate's job skills should continue to be completed as directed.

Process for Supervisors

1. Explain the Development Feedback process to your teammates.
2. Allow the teammate enough time to review the form and to consider the items you are asking him/her to complete.
3. Schedule time to meet with the teammate to review and discuss the document.
 - Schedule adequate time to meet.
 - Plan to meet in an area that is quiet and where you won't be interrupted.
 - Create a comfortable atmosphere.
 - Obtain additional feedback from your supervisor or Team Specialist on way to improve this important interaction with your teammates.
4. Refer to the "Development Feedback" completion instructions for information on how to complete the form.
5. Meet with each teammate within 90 days of hire and at least annually, or more frequently based on the specific action plans developed.
 - Review the Development Feedback tool from previous year and discuss progress made or changes to the plan.
6. Discuss and update the Development Feedback form with the teammate annually.

Worksheet: Human Resources & Staff Development

To complete this worksheet, you'll need access to the following:

- HR P&P Manual
- HR Forms Manual
- Education Resource Manual
- Access to the Portal

Activity	Results
1. Sign onto the Portal. Review the HR P&P Manual, Forms & Job Description Sections & Employee Handbook (as applicable)	Initial when complete: _____ Date: _____
2. Locate & review the HR P&P Manual and HR Forms Manual & read the following policies: • PTO & VHS • Harassment Free Workplace • Teammate Coaching • Clocking in/out meal breaks • Absenteeism • Drug & Alcohol Free Workplace	Initial when complete: _____ Date: _____
3. Review the facility Wage Scale. Verify it is up-to-date and each department is currently utilizing and strictly maintaining.	Initial when complete: _____ Date: _____
4. I-9 forms are required to be maintained in a separate binder from other teammate documents. Forms should be sorted between "current teammates" and "terminated teammates". Verify that this is in place.	Initial when complete: _____ Date: _____
5. Confirm that required Federal postings are current and posted in an open and easily accessible place for all teammates to see (e.g., near the time clock or in the teammate break area). Required postings may be included all information on one poster (referred to as a "5-in-1" poster). Information must include: • Job safety & Health Protection (OSHA) • Your Rights Under Family & Medical Leave Act • Equal Employment Opportunity is the Law • Uniform Services Employment & Re-employment Rights Act (USERRA) • Fair Labor Standards Act (FLSA)	Initial when complete: _____ Date: _____
6. List the state specific required postings for your facility & verify that they are current.	Initial when complete: _____ Date: _____
7. Does the facility have a union contract? If yes, review the contract and answer the following:	☐ Yes ☐ Not Applicable
a. What is the union name & local number?	☐ Not Applicable
b. Who are the covered teammates (this is normally found in the "Agreement & Recognition" clause/article)	☐ Not Applicable
c. When are the scheduled pay increases & what amount?	☐ Not Applicable
d. What is the expiration date of the contract?	☐ Not Applicable
8. Confirm that each department leader has a copy of the union contract	☐ Confirmed on _____ Initials: _____ ☐ Not Applicable

Worksheet: Human Resources & Staff Development

Activity	Results
9. Review the Applicant Flow Log and complete the following questions:	
a. Who maintains the Applicant Flow Log?	
b. Is the log current?	☐ Yes ☐ No, describe:
10. Review the OSHA log and answer the following questions:	
a. Who maintains the OSHA log?	
b. Is the OSHA log current?	☐ Yes ☐ No
c. Confirm that copies of the OSHA log are available in the Administrators office for the past 5 years	Initial when complete:_____ Date:_____
11. Review the Education Resource Manual and answer the following questions:	Initial when complete:_____ Date:_____
a. When is it appropriate to use "Quikstart" Orientation?	
b. Who completes and maintains the Orientation Records?	
c. What are the Teammate Health Requirements?	
12. Teammate Alert Line	Initial when complete:_____ Date:_____
a. What is the purpose of the hotline?	
b. How does the process work?	
c. What is your role & responsibility in responding?	

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Orientation Session #8

Compliance

Introduction

For this session, you'll focus on the systems & processes for compliance management in your facility. As part of this session you'll learn about:

- Compliance training
 - Code of Ethics and Business Conduct program
 - False Claims Act
 - Office of Inspector General (OIG)
 - Role of Facility Compliance Officer
- LaVie Alert Line for residents, families and teammates

To accomplish this session, you'll need the following resources and tools:

- Compliance Policy & Procedure Manual
- Access to the Compliance / HIPAA Portal Site

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the operations specialist from your management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do To Complete this Session

- ✓ Complete the required **Compliance Training P7003** at on-line university
- ✓ Complete the **Compliance Training Post Test**
- ✓ Demonstrate how to access & use the **Compliance site** on the Portal and the **OIG website**

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from your management company
- ✓ Complete "**Session #8**" of the **Orientation Checklist**

Orientation Session #8

Compliance

Compliance Training

As the Administrator of your facility, you are also the **Facility Compliance Officer**. It is your responsibility to ensure that all teammates receive compliance training upon hire and annually thereafter. A hundred percent validation must be completed at the end of each calendar year using the LMS system and letter of completion send to Compliance Committee.

A Compliance Awareness Training is available through the on-line university. You may use this content to teach the course live or ask your teammates to attend the course on-line.

The course name & number is **P7003 Compliance Awareness Training**. You'll need to complete this training as a new teammates. This is also the training that you can use to train your teammates.

The Compliance Awareness Training provides information on the following topics:

- Healthcare Oversight
- Code of Ethics and Business Conduct Program
- False Claims Act
- Duty to Report
- Healthcare oversight

Guidelines for Teaching the Course Live

1. Access the training program (P7003) through the on-line university.
2. Make a copy for each student
3. Present the slides and material live
4. Ask the students to complete the post test
5. Students must re-take the test if a score of 80% or greater is not achieved
6. Once 80% or greater is achieved, enter the student's score into the online university system via a "live up-load" form.

Guidelines for using the Online Course (*LaVie University*)

1. Meet with the student and discuss the purpose of the training.
2. Provide instructions for accessing the on-line university and in-service #P7003
3. Provide the student with access to a computer in the facility, preferably in a quiet location.
4. After the student completes the training, check their score to verify at least 80% was achieved. If not, the student must take the test again until 80% is achieved.

Orientation Session #8

Compliance

Accessing Resources

Compliance resources are available on the Portal.

- Access the Portal
- Click on “Depts”
- Click on “Compliance”
- View the documents available on the Compliance Portal
- Access the OIG Website from this page

LaVie Alert Line for Residents, Families and Teammates

LaVie Alert Line provides customers, their families and teammates with an avenue to express unresolved concerns and receive an acknowledgement call from the operations specialist of the management company within 3, 10 or 20 days depending on the nature of the call. Investigations of reported concerns are completed within 3, 10 or 20 days of receipt.

There are two convenient methods to report concerns: Compliance Alert Line – 877.631.5715 or the Compliance Website – www.laviecare.alertline.com. Concerns may be reported anonymously – 24 hours a day, 7 days a week. The facility has been supplied posters and postcards to display throughout the facility so this information can be shared with residents, family members and teammates.

RESIDENT/PATIENT, FAMILY & TEAMMATE ALERT LINE PROCESS

CURRENT ALERT LINE PROCESS UNDER REVISION

CONTACT CHIEF COMPLIANCE OFFICER WITH SPECIFIC QUESTIONS

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Orientation Session #9

Customer Satisfaction

Introduction

For this session, you'll focus on the systems & processes for promoting customer satisfaction. As part of this session you'll learn about:

- Our Mission
- Core Values
- "Commitment to Excellence" Customer Service Training
- Guardian Angel
- Grievance process
- Resident Council
- Family Council

To accomplish this session, you'll need the following resources and tools:

- Access to the Portal Site / Customer Service Training
- Administrative Manual
- Social Services Manual
- Recreation & Therapeutic Activities Manual

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the operations specialist from your management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #9

Customer Satisfaction

What to Do To Complete this Session

- ✓ Review the “**Commitment to Excellence**” program on the Portal
- ✓ Review the procedure & process for the “**Guardian Angel Program**” in the Administrative Manual
- ✓ Review the procedure & process for the “**Grievance Process**” in the Social Services Manual
- ✓ Review the procedure & process for “**Resident Council**” in the Recreation & Therapeutic Activities Manual
- ✓ Meet with the **President of Resident Council** to review issues/concerns.
- ✓ Review the procedure & process for the “**Family Council**” in the Social Services Manual
- ✓ Review Our Mission & Core Values
- ✓ Complete the “**Worksheet: Alert Line & Satisfaction Surveys**”

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from the management company
- ✓ Complete “**Session #9**” of the Orientation Checklist

Orientation Session #9

Customer Satisfaction

Our Vision

ResidentCare

- Quality of life
- Quality of care
- Trusting relationship

TeammateCare

- Respect
- Personal/professional growth
- Engaged/empowered/enthused



BusinessCare

- Census
- Cost
- Collections

How We Get There

Shared Mission

"People Taking Care of People...Together we can do anything"

Core Values

<i>Compassion</i>	We are present & responsive to the people we care for and with whom we work. We are caregivers who bring focus to "People Taking Care of People".
<i>Quality Commitment</i>	We deliver the highest quality care possible and continuously look for areas to improve.
<i>Service Excellence</i>	Serving others is what we are about. We strive with great integrity to understand our residents and teammates needs and exceed their expectations.
<i>Team</i>	Working together with a shared purpose and culture to achieve our goals and bring to life "Together We Can Do Anything".
<i>Fun</i>	We love what we do, even when we have hard days. We bring an upbeat and positive attitude to each session and the team.

Common Goals / Super 6

Care • Team • Census • Labor • Supply Cost • Cash Collections

Orientation Session #9

Customer Satisfaction

LaVie Customer Service Standards

- #1 The LaVie Team delivers world class customer service to everyone who visits the facility.
- #2 LaVie Teammates know and live LaVie Core Values—Team, Compassion, Quality Commitment, Service Excellence & Fun.
- #3 Every resident and family is welcomed warmly by all teammates. Department Leaders provide information and education on facility services and expectations.
- #4 Residents & Families are provided phone numbers to contact The Facility Administrator and Department Leaders as needed.
- #5 Each resident has a Guardian Angel. Guardian Angels are accountable to their residents, work hard to resolve issues and exceed expectations.
- #6 A Care Review meeting occurs with the resident and family within 72 hours of admission, as needed throughout the resident's stay to review the care plan and progress towards goals and 72 hours prior to discharge.
- #7 Residents and Families are provided information on the Grievance Process on admission. All teammates know how to recognize a complaint and how to communicate this information to their Facility Administrator.
- #8 A responsible leader is on duty every shift 24/7.
- #9 Resident Council & Family Council is promoted and encouraged. The Facility Administrator is responsive to recommendations and issues from the councils.
- #10 Monthly Town Hall/Team Meetings are held on all shifts.

Orientation Session #9

Customer Satisfaction

Commitment to Excellence / Customer Service Training

The Commitment to Excellence program is the customer service education and training program for all facility staff. This is part of orientation for all new teammates. It's recommended to complete the training program annually for all teammates.

Customer CARE drives the Commitment to Excellence training. CARE stands for:

- C** = Connect with your customer
- A** = Ask about your customers needs
- R** = Respond to your customers needs
- E** = Evaluate customer satisfaction

There are 4 modules to the program. They are:

- Module 1: "The Customer Connection"
- Modules 2 & 3: "Customer Care—Ask & Respond". These modules are combined
- Module 4: "Evaluating Customer Satisfaction"

Go to the Portal / Click on the sidebar "Clinical/Quality" Dept / Click on Orientation"/ Click on "Day One Orientation/ Click on "Customer Service Expectations"

You'll find PowerPoint presentations and handout materials for use in providing customer service training to your staff.

Orientation Session #9

Customer Satisfaction

Guardian Angel

The Guardian Angel is one of the processes used for routinely connecting with each individual resident/patient to find out how they are experiencing their stay at your facility, and what can be done to improve.

Selected staff, usually department managers, is assigned to be the “Guardian Angel” for a group of residents/patients. Each Guardian Angel is responsible for regularly interviewing their assigned residents/patients. Responses from the interviews are noted on a Guardian Angel form and reviewed at the Daily Operations Meetings.

Guardian Angels should be empowered to immediately respond and resolve identified issues if they can.

The Guardian Angel procedures are in the Administrative Manual. Read & review this process. The Guardian Angel forms are located on the portal.

Grievance Process

As the Administrator of your facility, you are responsible for the oversight of the grievance process. Your Social Services Director is usually the person responsible for the day-to-day acceptance of grievances and coordinating and verifying timely responses to those grievances.

Grievances must be responded to & resolved in a timely manner. Sometimes grievances may not be able to be resolved, but response and communication with the parties involved is critical to maintaining a positive relationship with the resident/patient, family and/or staff members involved in the grievance.

Develop a routine of reviewing grievances daily. Ask your Social Services Director to provide you with a status report daily. Pay attention to grievances that are taking days to resolve. **Your attention to resolving grievances quickly demonstrates strong leadership and sets a good example to your team.**

Review the “**Grievance Process**” procedure in the Social Services Manual.

Orientation Session #9

Customer Satisfaction

Resident Council

Resident Council is an excellent opportunity to listen and respond to the issues and concerns of your residents/patients. Get to know your Resident Council president and encourage the Resident Council to communicate with you frequently. Ask if you can attend their meetings. Respond to their issues. Even if you can't meet their request, take the time to explain the reasons why. Engaging with the Resident Council is an excellent opportunity to demonstrate your leadership and responsiveness.

The process for “**Resident Council**” can be found in the Recreation & Therapeutic Activities Manual.

Schedule time to meet with the **President of Resident Council**. Questions to consider asking include:

- Does staff listen to the resident council's views and act upon any grievances the resident group has filed?
- Does staff respond to the resident/group's concerns?
- Are there currently outstanding issues/concerns that have not been addressed?
- Do you feel the rights of residents are respected and encouraged?
- Do residents know who the ombudsman is for the facility and how to contact?
- Are residents aware of LaVie Alert Line and how to report concerns to management? If concerns are unresolved, are residents aware of how to contact the state?

Family Council

The Family Council is another opportunity to hear from your customers. Family Councils should be encouraged to bring ideas and concerns to you and your team. As with Resident Council, when issues are identified, you must respond. Satisfied families can become staunch supporters and will help you build a positive reputation in your community.

The process for “**Family Council**” can be found in the Social Services Manual.

Worksheet: Customer Satisfaction

Complete the following questions.

1. What is the Alert Line number for reporting complaints or concerns?	
2. Where is the Alert Line poster located in your facility?	
3. Who are the "Guardian Angels" in your facility?	
4. How is the Grievance Process managed in your facility?	
5. Who is the President of Resident Council?	
6. What concerns/issues does the President of Resident Council have?	
7. How often does Family Council meet?	
8. Are there any outstanding issues from Family Council meetings?	
9. When is the last time Customer Service Training was completed for all staff?	
10. What trends, issues or concerns have you identified from reviewing the Alert Line reports?	
11. What trends, issues or concerns have you identified from reviewing Alert Line reports?	
12. What trends, issues or concerns have you identified from reviewing customer satisfaction reports?	
13. Compare the trends, issues or concerns with the RM/QI Action Plan in your facility. Are improvement strategies aligned with the identified trends, issues and concerns? Is there anything that needs to be added or changed?	

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Orientation Session #10

Legal Issues & Risk Management

Introduction

For this session, you'll focus on the systems & processes for managing legal issues and risk management. As part of this session you'll learn about:

- Incident/Accident process
- Reporting requirements
- Identification & management of high risk residents/patients
- Legal support
- Contract Management
- Subpoena Requests
- Requests for Medical Records

To accomplish this session, you'll need the following resources and tools:

- Access to the Portal / Legal
- Access to the Portal / Risk & Insurance
- Administrative Manual
- Health Information Management Manual

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with operations specialist from your management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #10

Legal Issues & Risk Management

What to Do To Complete this Session

- ✓ Read the “**Abuse Prevention Program**” in the Administrative Manual
- ✓ Read the “**Incident/Accident**” process in the Administrative Manual
- ✓ Review the “**Incident Listing & Logging**” application on the Portal
- ✓ Review the “**Contract Review**” process in the Administrative Manual & on the Portal
- ✓ Review the “**Risk Management/Quality Improvement**” process in the Administrative Manual
- ✓ Attend the Risk Management/ Quality Improvement meeting in the facility & review focus areas for improvement

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from your management company
- ✓ Complete “**Session #10**” of the Orientation Checklist

Orientation Session #10

Legal Issues & Risk Management

Incident & Accident Management

An effective approach to risk management includes a thorough investigation and response to incidents & accidents.

- ✓ Read & review the “**Incident/Accident**” process in the Administrative Manual
- ✓ Review the “**Incident/Accident**” application on the portal.
Go to the Portal / Click on “Apps” / Select “Risk Mgmt” / Open the “Incident Listing & Logging”

Investigation tools are protected and confidential forms used as part of the Risk Management/Quality Improvement program. When logging an incident into the portal, an **Investigative Worksheet** and **Investigative Checklist** will be generated and available to print.

The **Investigative Worksheet** is an **internal document** and should be kept as part of the confidential Risk Management/Quality Improvement process.

The **Investigative Checklist** is a tool that demonstrates completion of the incident/accident investigation. This tool **may be shared** with regulatory agencies requesting to review.

Orientation Session #10

Legal Issues & Risk Management

Reporting Requirements

Report all allegations of abuse, neglect, exploitation, misappropriation and injuries of unknown source. Refer to the following definitions.

Term	Definition
Abuse	The <i>willful</i> infliction of injury, unreasonable confinement, intimidation, punishment with resulting physical harm, pain and/or mental anguish
Verbal abuse	Any use of oral, written or gestured language that <i>willfully</i> includes disparaging and derogatory terms to residents regardless of their age, ability to understand, or disability
Physical abuse	Includes hitting, slapping, pinching, kicking, etc. as well as controlling behavior through corporal punishment
Mental abuse	Includes, but is not limited to, humiliation, harassment, and threats of punishment or deprivation
Sexual abuse	Includes, but is not limited to, sexual harassment, sexual coercion, or sexual assault
Neglect	The failure to provide goods and services necessary to avoid physical harm, mental anguish, or mental illness
Exploitation/ Misappropriation	The deliberate misplacement, exploitation, or wrongful, temporary or permanent use of a resident's belongings without the resident's consent
Injuries of unknown source / origin	Meet both of the following criteria Source of injury not observed by anyone, or resident unable to explain source of injury AND Injury is suspicious because of the extent or location of the injury, or the number of injuries at one time or over time

Immediate Report

- Due within 24hrs of the event

Five Day Report

- Due within five (5) business days of the event
- Sent by fax or email
- Requires documentation of investigation findings; whether allegation substantiated or unsubstantiated; protective/corrective actions taken

Orientation Session #10

Legal Issues & Risk Management

Special reporting requirements for suspected criminal conduct:

The Elder Justice Act (EJA), subtitle H of the Affordable Care Act (ACA) requires “covered individuals” (defined as owner, operator, employee, manager, agent or contractor of a long-term care facility that receives \$10,000 or more in federal funds annually) to report to law enforcement authorities any reasonable suspicion of criminal activity against anyone who is receiving care from the facility.

- **Report MUST be made within 2 hours:** if the suspected criminal activity resulted in significant/ serious bodily injury, it must be reported immediately, but no later than 2 hours after the suspicion of criminal activity is formed.
- **Report MUST be made within 24 hours:** if the suspected criminal activity does not result in serious bodily injury, it must be reported within 24 hours of forming the suspicion.

High Risk Residents/Patients

Situations that create risk are inevitable for health care providers. Providing individualized and quality care and services to residents/patients reduces much of the risk, however, certain conditions may create a higher level of risk, and may require more time and focus to ensure the resident/patient receives the care and services needed.

High risk residents/patients may include, but are not limited to those that have:

- ✓ Skin and wounds
- ✓ Falls
- ✓ History or risk for elopement
- ✓ Multiple or unresolved complaints
- ✓ Unrealistic expectations
- ✓ Unplanned discharges

Refer to the “**Operations & Clinical Care Management**” process from Session #2 for information on clinical care meetings for residents/patients identified as higher risk.

Orientation Session #10

Legal Issues & Risk Management

Legal Support

Legal advice and support is available by contacting your management company.

Contact your management company **immediately** if you receive a subpoena request or a request for medical records.

Contract Management

All facility contracts should be submitted for review prior to being signed. The review process includes both a review by the legal department and a business review by the management company. This review process is designed to help ensure all contracts are legally appropriate, provide adequate protection to the facility and meet long-term financial objectives.

Read the “Contract Review” process in the Administrative Manual. This information is also available on the Portal. Access the Portal; Click on “Depts”; Click on “Legal”; Click on “Contract Review” requirements.

Requests for Medical Records

The Health Information Management Manual provides process and guidelines on how to manage a request for medical records. Federal regulations, like HIPAA, state that specific steps must be followed to ensure a resident/patient’s health information is protected. The manual provides guidelines and forms to assist in meeting these requirements.

Read & review the process **“Requests for Medical Records—Handling Of”** located in the Health Information Management Manual.

Subpoena Requests

Your facility contracts with a management company to provide guidance in the event that you receive a subpoena request for medical records. **Do not release information** until the subpoena is reviewed. Contact your management company for more information. Fax the request to the management company at 813-902-6181.

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Orientation Session #11

Information Technology

Introduction

For this session, you'll focus on information technology and your role and responsibility in supporting IT in your facility. As part of this session you'll learn about:

- Role of Administrator in assigning & managing user rights & access
- LaVie Information Technology Services
- Computer equipment
- Electronic Security & HIPAA compliance
- Portal applications /Key Factor Reports, Clinical Indicators, Cash, Expense Tracking System
- Security Officer Designation and Role

To accomplish this session, you'll need the following resources and tools:

- Access to the Portal

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the operations specialist from the management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do To Complete this Session

- ✓ Contact the LaVie IT Help Desk to request training. 1-800-830-4213

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from the management company
- ✓ Complete "Session #11" of the Orientation Checklist

Orientation Session #11

Information Technology

Security Officer Designation and Role

Introduction

Each HIPAA covered entity must designate its own Security Officer. It is the responsibility of the Facility Administrator to assume the role of facility Security Officer, or designate an appropriate member of facility senior management to assume that role. In addition, a back-up Security Officer must be designated and trained to fulfill all responsibilities. The backup Security Officer must be available at all times when the Security Officer is unavailable. Due to the strong inter-relation between Privacy and Security, in most instances, this person should be the same person designated to perform the facility Privacy Officer role.

Responsibilities

Key responsibilities of the facility Security Officer include the following:

- Implements, manages and enforces policies and procedures for locally performed HIPAA mandated security measures;
- Maintains liaison with company IT Security Officer on matters relating to technical and administrative safeguards;
- Leads information security awareness training initiatives for the workforce – initial orientation and subsequent annual training;
- Assures consistent use of system tools for the set-up and maintenance of user rights, unique login IDs and unique passwords;
- Maintains a process for facility reporting of any known or suspected security incidents and assure that all security incidents are reported to the IT Security Officer;
- Assures that any known or suspected data breaches, whether electronic or otherwise, are reported as security incidents, investigated, and reported in accordance with applicable data breach reporting requirements.
- Oversees compliance with physical safeguards in the facility aimed at maintaining confidentiality and privacy of PHI;
- Periodically monitors compliance with local security safeguards, policies and procedures.

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Orientation Session #12

Environment & Plant Operations

Introduction

For this session, you'll focus on the systems & processes for environmental management and plant operations. As part of this session you'll learn about:

- Disaster Plan
- Status of Plant Operations
- Equipment maintenance
- Environmental impact on quality of care

To accomplish this session, you'll need the following resources and tools:

- Administrative Manual
- Safety Manual (Located on TELs)
- Access to the TELS system

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities.

At the end of this session, review any questions you may have with the operations specialist from the management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

What to Do To Complete this Session

- ✓ Complete the "**Worksheet: Life Safety Review**"
- ✓ Review the current **TELS Exception Report** for the facility
- ✓ Complete **TELS Training** through Direct Supply
- ✓ Conduct a tour of the facility with the Maintenance Director and the management specialist.
- ✓ Confirm that the facility **Disaster Plan & Emergency Call List** is current
- ✓ Complete the **F323 Accident Prevention** training program on the portal
- ✓ Review the facility **Safety Manual**

When Sessions are Completed

- ✓ Review completed sessions with the operations specialist from the management company
- ✓ Complete "**Session #12**" of the **Orientation Checklist**

Orientation Session #12

Environment & Plant Operations

Disaster Plan

Locate the facility disaster plan and review. Call and confirm contracts and relationships with all vendors, transportation and evacuation centers.

Status of Plant Operations

Meet with the Maintenance Director to discuss the condition, concerns and repair schedules for the following:

- Roof
- Air conditioning
- Floors
- Fire Doors
- Sprinkler system
- Smoke detectors
- Kitchen equipment
- Water temperatures
- Laundry equipment
- Generator
- Resident care equipment (e.g., beds, scales, lifts, concentrators, side rails, IV pumps, enteral pumps, med carts, wheelchairs)
- Bathing equipment
- Furniture
- Verify Rental equipment in facility

Environmental Impact on Quality of Care

Review any outstanding work, maintenance or identified issues and evaluate for trends or potential concerns that may impact quality of care. Focus on areas that can reduce the risk of falls and other accidents. Determine if the facility already has quality improvement efforts underway, or if there are additional changes that can improve safety.

Complete **F323 Accident Prevention** training on the portal.

- Read the PowerPoint materials, CMS Documents and complete the post test
- Review any questions with the operations specialist

Orientation Session #12

Environment & Plant Operations

TELS

TELS is a web-based product that integrates federal safety standards into an on-line library of Life Safety tasks. These tasks automatically schedule Life Safety tasks and prompts facility staff to record the required documentation that should be reviewed during monthly QA meeting. TELS allows facilities to track warranty details for equipment and record purchase dates & repairs.

Review the “How to Use Your Work Schedule” information from TELS to obtain and review the **TELS Exception Report**

Contact Direct Supply to schedule TELS Training at 888-367-3504 or 800-250-8273

Worksheet: Life Safety Review

Date: _____ Completed with: _____

Areas to Review	Findings
1. Review Life Safety citations and/or recommendations from the last two surveys. Identify any unresolved issues.	
2. Review results of the last Fire Marshall inspection. Identify any unresolved issues.	
3. Review contracts for Life Safety Preventive Maintenance. Confirm services are being performed as contracted. Identify any issues.	
4. Confirm schedule and occurrence of Fire Drills for this month. Note the schedule for all shifts including weekend shifts.	
5. Confirm fire extinguishers are checked monthly by maintenance directors. Checks should include proper pressure, locking pin in place and no damage	
6. Fire extinguishers should be serviced by a certified contractor once a year. What is the date of the last yearly service by the Fire Extinguisher company?	
7. Confirm Fire Alarm Testing is followed. Determine the following dates:	
Weekly pressure gauge reading: street side, facility side and valves opened & locked	
Date of last Quarterly Flow Test	
Date of last yearly Fire Alarm Test by Alarm Company	
Date of last smoke detector sensitivity test	
Date of last semi-annual kitchen ansul test	
8. Confirm Weekly Generator Testing is completed & includes documentation of oil pressure, fuel level, water temp, voltage output, amperage output, hertz and engine hours. Once test per month must be under full load.	
Name of Company who services generator	
Date of last generator service	
9. Confirm water temperatures in resident areas are tested & logged daily, including shower & bathing facilities. Review temperature logs. Temps should not exceed 110 degrees Fahrenheit	
10. Confirm fire & smoke doors are checked monthly	
11. Confirm dryers, vents and ductwork is cleaned bi-monthly	

Worksheet: Life Safety Review

Areas to Review	Findings
12. Confirm Nurse Call System is checked weekly. Include bulbs, cords and sounding devices. Include all call stations, resident rooms, bathrooms and common area devices	
13. Confirm wander prevention system or magnetic door locks are inspected & tested weekly. Check codes. Magnetic locks should release after 15 seconds of pressure and release during fire alarms.	
14. Confirm inspection and repairs have been done to smoke and fire barrier walls and ceilings.	
15. Confirm yearly boiler inspections are done. Note last inspection date	
16. Confirm scale inspections and calibrations are completed monthly	
17. Confirm electric lifts are checked monthly	
18. Confirm HVAC filters are checked monthly	
19. Confirm HVAC coils are cleaned quarterly	
20. Confirm PTAC filters are cleaned monthly	
21. Confirm exhaust fans are cleaned monthly	
22. Confirm kitchen equipment is inspected quarterly	
23. Confirm roof, gutter and roof drains are inspected monthly	
24. Confirm company van or bus inspections are completed weekly	
25. Confirm exterior grounds are inspected weekly	
26. Confirm emergency battery lighting is checked monthly	
27. Confirm exterior lighting is checked monthly	
28. Confirm exit signs are checked monthly	
29. Visualize the generator, generator enunciator panel and fire panel	
30. Confirm all permit guides are current and visual inspections have been completed.	

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Orientation Session #13

Regulatory Compliance

Introduction

For this session, you'll focus on the systems & processes for regulatory compliance in your facility. As part of this session you'll learn about:

- Facility survey history
- Local survey office & surveyors
- Survey preparation & setting expectations
- Traditional survey vs. QIS survey
- Impact of regulatory non-compliance
- Tag files
- Survey notification/communication
- Publicly reported survey outcomes

To accomplish this session, you'll need the following resources and tools:

- Access to the Portal
- Access to the Internet

Learning activities have been provided to assist you in developing the key skills needed for your role as the Administrator. Review & complete the learning activities. As you complete, record any questions you may have.

At the end of this session, review any questions you may have with the clinical and operations specialist from the management company and fill out the **Orientation Checklist** to indicate you've completed each step of the orientation process.

Orientation Session #13

Regulatory Compliance

What to Do To Complete this Session

- ✓ Complete the “**Worksheet: Long Term Care Survey**”
- ✓ Review survey results for the past 3 years
- ✓ Send a letter to the local survey office, notifying of new Administrator
- ✓ Visit the local survey office / meet survey team
- ✓ Obtain & review publicly reported data (Nursing Home Compare, 5-Star Program, Quality Measures)

When Sessions are Completed

- ✓ Review completed sessions with the clinical and operations specialist from the management company
- ✓ Complete “**Session #13**” of the **Orientation Checklist**

Orientation Session #13

Regulatory Compliance

The Survey Process

The successful operation and management of a skilled nursing facility is dependent on survey process. Facilities who receive Medicare and Medicaid funding are required to maintain substantial compliance with State and Federal regulations.

Federal regulations are published in the Federal Register. CMS publishes what is known as “Interpretive Guidelines” to these Federal Regulations. These guidelines form the basis for how surveyors look at how you provide clinical care in your facility to determine compliance.

Federal Regulations and the corresponding Interpretive Guidelines can be found at www.cms.gov

Each state has its own regulations for skilled nursing facilities. Some states are more detailed and specific in regulatory content than others. Each state publishes regulations online and they are available to the public. Work with your clinical and operations specialist to find out how to obtain a copy of your state’s regulations for long term care or skilled nursing facilities. Often, a state may publish additional regulatory guidance, or codes to provide greater insight in how to meet the intent of the regulation.

If the situation arises where Federal and State regulations don’t align, always adopt and follow the regulation that is more stringent.

The nursing department is responsible for the quality of care provided to each resident and many of the regulations are directly and indirectly linked to the nursing department.

The key to a successful survey is preparation, knowledge about the residents and facility processes, and supporting documentation of compliance.

How to Prepare for Survey

There is a saying, “Always be Survey Ready”. If your clinical team consistently follows policies and procedures, consistently provides quality care, consistently address outcome variances, issues and concerns quickly and professionally, then you will be survey ready.

Orientation Session #13

Regulatory Compliance

Frequently Asked Questions

What should I do when surveyors are in the facility?

The most important thing is, don't panic. As the Administrator you must present in an organized, confident manner. The facility should manage the survey process from entrance to exit. The followings tips have proven useful:

- Have a survey readiness notebook (that contains all documents the surveyors will request) ready to give surveyors during entrance conference
- Have staff trained and prepared to tour with surveyors.
- Identify the facility system for notifying all staff that surveyors are in the facility.
- If staff members provide surveyors with any documents instruct them to make a copy for facility and keep all documents together.
- Department leaders should be prepared to arrive early and stay until surveyors exit for the day.
- Instruct staff to report all conversations and interactions with surveyors so you are able to monitor the survey process.
- Check in with team leader regularly, inquire if there is anything you can do for them or help them obtain information they need.
- Your specialists are available via phone or will be at the facility during survey if you should need assistance.

Orientation Session #13

Regulatory Compliance

What is a tag file?

A tag file is a folder labeled and used to collect and organize proof of correction when a citation has been received. When the facility has a survey resulting in citations a separate file will be established for each citation. The file for each citation should include the following:

- A copy of the actual citation printed on the 2567 including the POC. Address how corrective action will be accomplished for those residents found to have been affected by the deficient practice;
- Proof addressing how the facility will identify other residents having the potential to be affected by the same deficient practice;
- Proof of measures that have been put in place or systemic changes made to ensure that the deficient practice will not recur;
- Proof of the facility plans to monitor its performance to make sure that solutions are sustained. The facility must develop a plan for ensuring that correction is achieved and sustained. This plan must be implemented, and the corrective action evaluated for its effectiveness. The plan of correction is integrated into the quality assurance system; Include dates when corrective action will be completed. The corrective action completion dates must be acceptable to the State.

What are some suggestions for writing an effective Plan of Correction (POC)?

A written POC is required in response to citations received during a survey. Surveyors send a “2567” which outlines the citations and the surveyors’ evidence for citing. Your responsibility is to write a response to these citations in the form of a “plan of correction”.

As part of his/her services, a nurse specialist will review the completed 2567 before it is turned into the area office. The specialist is available to assist the facility in developing the POC.

A well-written POC addresses issues directly, while supporting the hard work performed on a daily basis. The following items are a few tips for writing an effective Plan of Correction:

- Write a disclaimer on the front page of the CMS-2567 to remind readers that there are two sides to every story. Here is a sample disclaimer that is commonly used:

This Plan of Correction constitutes my written allegation of compliance for the deficiencies cited. However, submission of this Plan of Correction is not an admission that a deficiency exists or that one was cited correctly. This Plan of Correction is submitted to meet requirements established by state and federal law.

Orientation Session #13

Regulatory Compliance

- Write a facility policy statement in reference to the cited deficiency, and thoroughly outline the corrective action.
Example: Facility is cited for F314 – failure to prevent the development of pressure ulcers. Begin the POC with –It is the policy of the facility to promote the prevention of pressure ulcers.
- Describe the corrective actions taken for the resident(s) cited in the deficiency
- Describe how other residents potentially affected will be identified
- Detail any specific measures or systematic changes implemented to prevent reoccurrence
- Detail who (what position) will be responsible for monitoring compliance
- Discuss how the corrective measures will be reported to and monitored in the facility RM/QI committee

Common mistakes when writing a POC include the following:

- Stating that a teammate was “disciplined” which implies guilt.
- Excluding language that would support and/or defend the facility’s approach.
- Insufficient response to the basic federal requirement of a POC.

What is a QIS survey?

QIS stands for Quality Indicator Survey. The QIS survey process will eventually replace the traditional survey process. Not all states are actively participating at this time. The QIS is a two-staged, computer-assisted survey process. Resident sampling and triggers for further investigation are different from the traditional survey process. The QIS process does not change the scope and severity or the citation process. Your management specialist will provide you with information and training as appropriate for your survey area.

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Regulatory Compliance

What is the impact of regulatory non-compliance?

The impact of regulatory non-compliance can range from a minor inconvenience, to significant fines and penalties that can affect facility operations. When a facility is in non-compliance with regulatory requirements, a timeline starts. If compliance is not achieved within a certain timeframe, a facility may be terminated from the Medicare program.

In addition to the Federal enforcement process, the state of Florida has an additional track for enforcement that was added with the passage of SB 1202 in 2002. SB 1202 provides the state with guidance to apply enforcement penalties in addition to federal CMS penalties. SB 1202 provided alignment of state citations classification with federal scope and severity. A conditional license may be issued for citations classified as Class I, II, or uncorrected Class III. Each classification has associated required fine amounts.

A conditional status indicates that a facility did not meet, or correct upon follow-up, minimum standards at the time of a standard or complaint inspection. Immediate action is taken if a facility poses a threat to resident health or safety.

Facilities are notified of conditional license, citations and fines via an Administrative Complaint letter.

What are CMPs?

CMP is short for a “Civil money penalty”. A CMP is proposed following a survey that results in certain types of citations. Determining the actual CMP amount is based on the following:

- Use of scope and severity (as that matrix has been applied under the existing enforcement rule) to assist in determining the magnitude of the noncompliance, including whether actual harm has occurred.
- The facility’s degree of culpability.
- The facility’s history of prior offenses, including repeat deficiencies.

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Regulatory Compliance

CMS State Operations Manual & the Federal Register (42 CFR Part 488) outlines the following:

§ 488.430 Civil money penalties: Basis for imposing penalty.

CMS or the State may impose a civil money penalty for the number of days that a facility is not in substantial compliance with one or more participation requirements, or for each instance that a facility is not in substantial compliance, regardless of whether the deficiencies constitute immediate jeopardy. Additionally, CMS or the State may impose a civil money penalty for the number of days of past noncompliance since the last standard survey, including the number of days of immediate jeopardy.

REMEDY CATEGORIES		
Category 1 (Cat.1)	Category 2 (Cat.2)	Category 3 (Cat.3)
Directed Plan of Correction	Denial of Payment for New Admissions	Temp. Mgmt. Termination
State Monitor; and/or Directed In-Service Training	Denial of Payment for All Individuals imposed by CMS; and/or Civil money penalties: \$50 - \$3,000/day \$1,000 - \$10,000/instance	Optional: Civil money penalties 3,050-\$10,000/day \$1,000 - \$10,000/instance

What is DPNA?

Denial of Payment for New Admissions is a type of enforcement remedy that is imposed on a nursing home for failure to comply with Medicare and/or Medicaid certification requirements. In general, DPNA is imposed when a nursing home has serious deficiencies, a poor compliance history, or has failed to correct previously cited deficiencies.

When imposed, a facility is ineligible to receive Medicare and/or Medicaid payments for new admissions to the facility. The sanction is lifted after the facility achieves substantial compliance with the certification regulations.

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Regulatory Compliance

What is a Directed Plan of Correction?

A Directed Plan of Correction is a remedy that may be imposed based on the seriousness of the deficiencies. CMS, the State Survey Agency, or a temporary manager (with CMS or State approval) may develop a plan of correction and require a facility to take action within specified timeframes.

What are the publicly reported survey outcomes?

CMS publishes Nursing Home Compare at www.medicare.gov ;This site provides information on recent surveys, staffing and quality measures. In December 2008, CMS released a new “5-Star” rating system as a part of Nursing Home Compare. The number of “stars” a facility receives is based on a computation of citations received during the last 3 standard surveys, citations received during complaint surveys, ratio of RN and overall staffing and Quality Measures.

CMS also publishes the Special Focus Facility (SFF) list. This is a list of facilities with the most serious deficiencies that have been unable to regain sustainable regulatory compliance.

Review your facility’s “5-Star” status on nursing home compare and study the areas where improvement may be needed.

Worksheet: The Long Term Care Survey

- 1) Do you have a copy of "The Long Term Care Survey" manual published by AHCA?
Yes____ No____ If so what edition is it? _____
- 2) When was the last standard survey conducted for the facility? _____
- 3) When does the standard survey window open for the facility? _____
- 4) What does the term F tag mean? _____
- 5) A deficiency is?

- 6) When surveyors suspect substandard quality of care they _____ to determine scope.
- 7) A standard survey must be conducted at a minimum of every _____ months.
- 8) The standard window opens at _____ months after the entrance date of the last standard survey.
- 9) There are 4 severity levels for federal citations.
 - Level I = _____
 - Level II = _____
 - Level III = _____
 - Level IV = _____
- 10) Scope has 3 levels :
 - Isolated _____
 - Pattern _____
 - Widespread _____
- 11) If a facility does not clear citations and is out of compliance for 90 days the facility will face:
DPNA _____ and possible fines.

Worksheet: The Long Term Care Survey

Locate the following F tags and identify the regulation:

F tag #	Regulation – brief description of area of focus
F 155	
F 221	
F 223	
F 309	
F 314	
F 323	
F 329	
F 353	
F 387	

NOTES